

THE SUNNAH REMEDIES ACADEMY FRAMEWORK

Definitive Institutional Blueprint — Version 3.0

Institute of Prophetic Medicine

Classification: PROPRIETARY & CONFIDENTIAL **Effective Date:** July 2026 **Review Cycle:** Annual **Governance:** Sunnah Remedies Academy Academic Board

“This framework is the foundation upon which every Sunnah Remedies course is built. It is not a course outline. It is the architecture of an educational ecosystem — the definitive standard for a premium international Islamic academy of Prophetic Medicine.”

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PART ONE: THE ACADEMY ECOSYSTEM

1.1 Institutional Identity & Brand Architecture

The Unified Institution

Sunnah Remedies is a single, premium institutional brand expressed through four interconnected pillars. Every educational product, clinical service, and community initiative reinforces one coherent institutional experience — never fragmented sub-brands.

Pillar	Function	Audience
The Institute	Education, certification, research, professional standards	Students, practitioners, healthcare partners
The Clinic	Clinical services, patient care, practical training venues	Patients, students in supervised practice
The Apothecary	Prophetic remedies, supplies, equipment, formulary	Practitioners, patients, retail
The Community	Alumni network, CPD events, research collaboration, mentorship	Graduates, partners, scholars

Brand Identity Parameters

The register is deliberately secular-luxury: quiet, exact, and confident. Benchmarked against Aesop, Aman, Jo Malone, and Harvard Executive Education — never faith-institutional or clinical-medical conventions. Restraint is the defining discipline.

Three Colours Only:

Colour	HEX	RGB	CMYK	Role
Deep Emerald	#0E3B2E	14, 59, 46	84, 45, 68, 52	Primary —ground, authority, depth
Antique Gold	#C7A25A	199, 162, 90	22, 34, 72, 3	Accent only — emphasis, sacred connection
Warm Ivory	#F6F3EE	246, 243, 238	3, 3, 6, 0	Ground, air, readability

Typography:

- **Libre Baskerville** — display, headings, titles, moments of authority. Print-rooted serif signalling scholarship.
- **Inter** — body text, labels, captions, interface. Precise neutral sans signalling modern clarity.

Principles:

- One design system. One navigation model. One institution.
- Restraint over decoration — every element must earn its place.
- Real substance carries the brand: named authorship, dated work, credentials, citations.
- Distinctiveness comes from print-craft typography and genuine institutional detail, not decoration.

1.2 The Four Pillars

The Institute (Education)

The academic engine of Sunnah Remedies. Responsible for:

- Curriculum design, development, and continuous improvement
- Student admissions, progression, and certification
- Faculty development and teaching quality
- Research output and evidence synthesis
- Accreditation maintenance and external partnerships
- Quality assurance and academic governance

The Clinic (Clinical Services)

The clinical practice arm. Responsible for:

- Direct patient care at the highest standards
- Practical training venues for supervised clinical hours
- Clinical case generation for teaching and research
- Equipment testing and protocol validation
- Community health outreach
- Practitioner placement and internship coordination

The Apothecary (Remedies & Supplies)

The supply and formulary arm. Responsible for:

- Sourcing and quality-testing Prophetic remedies (honey, black seed, olive oil, etc.)
- Equipment supply: cups, pumps, lancets, PPE, sterilisation equipment
- Formulary development: evidence-informed remedy protocols
- Retail to practitioners and the public
- Supply chain ethics: halal, organic, fair-trade sourcing where applicable

The Community (Network & Continuity)

The relational and continuing education arm. Responsible for:

- Alumni network management and engagement
- CPD programme delivery
- Research collaboration and publication support
- Mentorship matching (senior practitioners with new graduates)
- Annual conference and regional gatherings
- Online community platform and knowledge sharing

1.3 Educational Ecosystem: Course Catalogue

The SRA Numbering System

Every Sunnah Remedies Academy course carries a unique identifier: **SRA-XXX** (Sunnah Remedies Academy — Course Number).

Current Course Portfolio

Code	Course Title	Level	Hours	Award	Prerequisites
SRA-001	Certified Hijama Practitioner	Foundation	160	CHP	None (age 18+, English literacy)

Table 3 – continued

Code	Course Title	Level	Hours	Award	Prerequisites
SRA-002	Advanced Hijama & Clinical Applications	Advanced	80	AHP	SRA-001 + 100 practice hours
SRA-003	Prophetic Medicine Foundations	Foundation	60	PMF	None
SRA-004	Herbal Medicine & Materia Medica	Intermediate	80	HMM	SRA-003 recommended
SRA-005	Nutrition in Prophetic Medicine	Intermediate	60	NPM	None
SRA-006	Ruqyah & Spiritual Wellbeing	Foundation	60	RSW	None
SRA-007	Women’ s Health in Prophetic Medicine	Intermediate	80	WHP	SRA-001 or SRA-003
SRA-008	Sports Recovery & Performance Hijama	Advanced	60	SRP	SRA-001 + 50 practice hours
SRA-009	Instructor Certification Programme	Expert	120	SRA-IC	SRA-001 + 200 hours + 2 years practice

Future Course Pipeline (2027–2030)

Code	Course Title	Level	Projected Launch
SRA-010	Paediatric Applications of Prophetic Medicine	Advanced	2027 Q3
SRA-011	Hijama for Chronic Disease Management	Advanced	2028 Q1
SRA-012	Research Methods in Prophetic Medicine	Expert	2028 Q2
SRA-013	Clinic Management & Healthcare	Intermediate	2028 Q4
SRA-014	Entrepreneurship Integrative Medicine: Bridging Conventional & Prophetic	Expert	2029 Q2

Certification Pathway Architecture

FOUNDATION	INTERMEDIATE	ADVANCED	EXPERT
SRA-003 Prophetic Medicine Foundations	SRA-004 Herbal Medicine & Materia Medica	SRA-002 Advanced Hijama & Clinical Apps	SRA-009 Instructor Certification
SRA-001 Certified Hijama Practitioner	SRA-005 Nutrition in Prophetic Medicine	SRA-008 Sports Recovery & Performance Hijama	SRA-012 Research Methods in Prophetic Medicine
SRA-006 Ruqyah & Spiritual Wellbeing	SRA-007 Women's Health in Prophetic Medicine		SRA-014 Integrative Medicine
MASTER PRACTITIONER PATHWAY (SRA-MP)			
SRA-001 + SRA-002 + SRA-003 + SRA-004 + SRA-005 + 500 supervised hours + Research project			

1.4 World-Class Benchmarking Framework

Benchmark Institutions

Sunnah Remedies Academy measures itself against the highest international standards across multiple domains:

Domain	Benchmark Institution	What We Adopt	What We Adapt
Curriculum Structure	Harvard Medical School	Systems-based, integrated curriculum; case-based learning; early clinical exposure	Islamic scholarship integration; Prophetic Medicine foundations
Clinical Training	Mayo Clinic Model of Care	Patient-centred, evidence-informed, team-based; rigorous competency assessment	Integration of Tibb al-Nabawi principles with clinical reasoning
Assessment Rigor	Royal College of Physicians (UK)	OSCE design, portfolio assessment, continuing recertification	Islamic ethics stations; Sunnah-based clinical reasoning scenarios
Evidence Standards	NICE (National Institute for Health and Care Excellence)	Systematic review methodology; honest communication of uncertainty; GRADE criteria	Application to traditional and Prophetic Medicine interventions

Table 5 – continued

Domain	Benchmark Institution	What We Adopt	What We Adapt
Islamic Scholarship	Al-Azhar University; Oxford Centre for Islamic Studies	Rigorous authentication of sources; classical scholarly methodology; Arabic text verification	Integration with clinical practice and modern scientific literacy
Professional Standards	GMC (General Medical Council, UK)	Code of conduct; fitness to practise; professional boundaries; patient safety	Islamic ethical framework (Maqasid al-Shari' ah) as foundational
Visual & Educational Design	Harvard Executive Education; Apple	Premium visual quality; information architecture; restraint and precision	Warmth and human connection appropriate to healing professions
Research Integration	Johns Hopkins Medicine	Evidence-based practice culture; research literacy; continuous updating	Inclusion of traditional medicine research; cross-cultural evidence

Academy Standards Alignment Matrix

SRA Standard	Benchmark Source	SRA Enhancement
SRA-TS-001 Teaching	Harvard Medical School CME	Islamic pedagogical tradition (talab al-' ilm)
SRA-CPS-001 Clinical Practice	NICE Guidelines + WHO	Maqasid al-Shari' ah clinical decision framework
SRA-AS-001 Assessment	Royal College OSCE Model	Faith-integrated clinical scenarios
SRA-PCS-001 Professional Conduct	GMC Good Medical Practice	Islamic ethics of healing (amana, ihsan, rahmah)
SRA-CF-001 Competency	CanMEDS Framework	Spiritual competency dimension
SRA-GA-001 Graduate Attributes	AAC&U VALUE Rubrics	Islamic intellectual tradition integration
SRA-CE-001 Code of Ethics	UNESCO Bioethics	Maqasid al-Shari' ah bioethical framework
SRA-CEF-001 Clinical Excellence	IHI Triple Aim + Donabedian	Spiritual wellbeing as fourth aim
SRA-RMPS-001 Risk Management	NHS National Patient Safety Agency	Islamic principles of harm prevention (darar)
SRA-CPD-001 CPD	Royal College CPD Framework	Islamic continuing education (talab al-' ilm)

1.5 Mission, Vision & Graduate Profile

Mission

To restore Prophetic Medicine to its rightful place in the global healthcare landscape — producing practitioners who are simultaneously clinically excellent, spiritually grounded, and intellectually rigorous.

We exist to:

- **Educate** practitioners who bridge sacred tradition and scientific evidence with intellectual honesty
- **Elevate** the standards of Hijama and Prophetic Medicine practice worldwide
- **Contribute** to the global evidence base through original research and critical synthesis
- **Preserve** authentic Islamic scholarly tradition while embracing methodological progress
- **Serve** humanity through healing practices rooted in revelation and refined by reason

Vision

Within ten years, Sunnah Remedies Academy will be:

- **The global reference** for Hijama education — the institution against which all other programmes are measured
- **A research contributor** with peer-reviewed publications in cupping therapy, Prophetic Medicine, and integrative health
- **A partnership network** spanning universities, hospitals, Islamic institutions, and regulatory bodies across six continents
- **A living curriculum** that evolves with scientific discovery while honouring unchanging revelation
- **A community** of practitioners united by excellence, integrity, and service

Graduate Profile — The SRA Practitioner

Upon certification from any SRA programme, a graduate will demonstrate:

Clinical Excellence (Itqan)

- Technical mastery of all procedures taught, performed with precision and care
- Systematic clinical reasoning from assessment through treatment to follow-up
- Uncompromising safety standards and infection control discipline
- Recognition of personal limits and appropriate referral pathways
- Competent management of common complications and emergency recognition

Intellectual Rigor (Ilm)

- Critical appraisal of research literature with understanding of methodological strengths and limitations
- Honest communication of evidence to patients — what is known, what is uncertain, what is traditional
- Distinguishing between Qur'anic teaching, authentic Hadith, scholarly interpretation, traditional practice, and scientific evidence
- Continuous learning mindset and commitment to evidence-based evolution

Spiritual Grounding (Tazkiyah)

- Sound understanding of the Islamic foundations of their practice
- Integration of Islamic ethical principles into every clinical decision
- Appropriate communication of spiritual dimensions to patients of all backgrounds
- Personal commitment to the spiritual dimensions of healing — recognising Allah as the ultimate Healer

Professional Integrity (Amanah)

- Practice within scope of competence and regulatory framework
- Ethical marketing — never making unsubstantiated claims
- Respect for patient autonomy, dignity, and confidentiality
- Interprofessional collaboration and appropriate referral
- Commitment to continuing professional development

Compassionate Care (Rahmah)

- Genuine compassion as the foundation of every patient interaction
- Patient-centred communication that listens, explains, and respects
- Cultural sensitivity and ability to serve diverse communities
- Understanding that every patient is an amanah — a sacred trust

1.6 Core Values & Maqasid al-Shari'ah Integration

The Five Core Values

Ihsan — Excellence in Everything

“Indeed, Allah loves those who do good (al-muhsinun).” — Qur'an 2:195

We do not accept “good enough.” Every lesson, every clinical protocol, every patient interaction is crafted to the highest standard. The Prophet’s instruction to “do things with excellence” (hadith: inna Allaha yuhibbu idha 'amila ahadukum 'amalan an yutqinahu) applies to education as much as to practice.

Amanah — Trustworthiness

“Indeed, We offered the Trust (al-Amanah) to the heavens and the earth and the mountains, and they declined to bear it. But mankind undertook it.” — Qur'an 33:72

Our students trust us with their education, their time, and their aspirations. Our patients trust our graduates with their health and their bodies. We honour that trust with accuracy, honesty, and rigour. We never teach what we cannot verify.

Rahmah — Compassion

“And We have not sent you, [O Muhammad], except as a mercy to the worlds.” — Qur'an 21:107

Every patient who enters a Sunnah Remedies graduate’s clinic will be treated with dignity, gentleness, and genuine care. Compassion is not an add-on — it is the foundation. The Prophet’s mercy is our model for patient care.

Ilm — Knowledge with Humility

“And say, ‘My Lord, increase me in knowledge.’” — Qur'an 20:114

We pursue knowledge relentlessly — both the revealed knowledge of Islam and the discovered knowledge of science. We hold both with humility, recognising that all true knowledge comes from Allah. We reject both scientism (the rejection of revealed knowledge) and anti-intellectualism (the rejection of scientific inquiry).

Itqan — Mastery through Craft

“That they may witness benefits for themselves.” — Qur’an 22:28

We believe that mastery comes through deliberate practice, continuous refinement, and attention to detail. Our graduates are craftsmen of clinical care — every incision precise, every assessment thorough, every interaction intentional.

Maqasid al-Shari’ah Integration

The objectives of Islamic law (Maqasid al-Shari’ah) provide the ethical architecture for all SRA programmes. These five universal objectives, classically articulated by Imam al-Ghazali and developed by scholars including al-Shatibi, inform every curriculum decision, clinical protocol, and ethical guideline.

Maqsad (Objective)	Preservation Of	Curriculum Integration
Hifz al-Din	Religion	Islamic scholarship modules; faith-integrated clinical reasoning; spiritual dimensions of healing
Hifz al-Nafs	Life	Patient safety as paramount; contraindication mastery; emergency procedures; risk management
Hifz al-’ Aql	Intellect	Evidence literacy; critical thinking; honest communication of uncertainty; research methodology
Hifz al-Nasl	Lineage	Women’ s health modules; family wellness; reproductive health; paediatric considerations
Hifz al-Mal	Property	Ethical business practice; fair pricing; honest marketing; professional indemnity; legal compliance

Every clinical protocol, assessment item, and ethical guideline is evaluated against these five objectives. No SRA graduate will be certified without demonstrating understanding of how their practice serves the Maqasid al-Shari’ah.

1.7 The Complete Student Journey

Phase 1: Discovery & Enrolment (Pre-Course)

Touchpoint	Deliverable	Purpose
Initial Inquiry	Premium course brochure (digital + print)	Communicate value, rigour, and outcomes
Information Session	Live webinar with faculty	Build relationship, answer questions, set expectations
Application	Online application with prerequisite verification	Ensure readiness and fit

Table 8 – continued

Touchpoint	Deliverable	Purpose
Admission Decision	Personalised acceptance communication	Create sense of belonging
Enrolment Confirmation	Welcome pack (see below)	Prepare student for success

The SRA Welcome Pack

Every enrolled student receives:

- **The SRA Student Handbook** — programme guide, policies, expectations
- **The SRA Learning Journal** — structured reflection journal for the programme
- **Equipment Starter Kit** — clinical supplies for practice (SRA-001 specific)
- **Anatomy Reference Cards** — laminated quick-reference sheets
- **Access Codes** — online learning platform, research databases, Islamic reference library
- **Personalised Schedule** — their specific cohort timetable
- **Welcome Letter** — from the Academic Director, personally addressed

Phase 2: Orientation (Week 0)

Activity	Duration	Purpose
Online Orientation Module	Self-paced (2 hours)	Platform familiarisation; study skills; academic integrity
Live Orientation Session	2 hours	Meet cohort; meet faculty; Q&A; set intentions
Clinic Tour (in-person cohorts)	1 hour	Familiarisation with training venue
Buddy Assignment	Ongoing	Peer support pairing
Personal Learning Plan	30 min	Goal-setting with mentor

Phase 3: Active Learning (Course Duration)

Element	Description
Live Lectures	Expert-led with premium visuals, interactive elements, Q&A
Practical Workshops	Hands-on supervised practice with individual feedback
Case Studies	Realistic clinical scenarios worked in groups
Knowledge Checks	Formative assessment after every lesson
Reflection Prompts	Islamic-integrated self-assessment in learning journal
Progress Tracking	Digital dashboard showing completion, scores, competencies

Table 10 – continued

Element	Description
Mentor Check-ins	Bi-weekly 15-minute individual mentor sessions
Peer Learning Groups	Small group study and practice sessions
Guest Lectures	Visiting scholars, clinicians, researchers

Phase 4: Assessment (Post-Teaching)

Assessment	Timing	Support Provided
Written Examination	Week after final lesson	Revision guide; mock exam; study groups
Practical OSCE	1-2 weeks after written	Practice stations; peer rehearsal; coaching
Portfolio Submission	2-4 weeks after OSCE	Portfolio template; exemplars; supervisor guidance

Phase 5: Certification & Transition

Element	Description
Results & Feedback	Detailed feedback report with strengths and development areas
Certification Ceremony	Formal presentation of credentials (live or virtual)
Alumni Onboarding	Transition to SRA Community; CPD pathway introduction
First-Year Mentorship	Monthly check-ins with senior practitioner mentor
Practice Support Pack	Clinic setup guide; marketing templates; insurance guidance

Phase 6: Alumni & CPD (Ongoing)

Element	Description
Alumni Community Platform	Online forum; case discussion; resource library
CPD Events	Monthly webinars; quarterly workshops; annual conference
Advanced Pathways	Progression to SRA-002, SRA-008, SRA-009
Research Participation	Opportunities to contribute to SRA research programme
Mentorship Programme	Senior graduates mentor new practitioners
Recertification	Every 2 years via CPD portfolio review

Advanced Practitioner Pathways

CHP (SRA-001 Graduate)

- └─> Advanced Hijama Practitioner (AHP) — SRA-002
 - └─> Master Practitioner (SRA-MP) — Research + 500 hours
- └─> Sports Recovery Specialist — SRA-008
- └─> Women's Health Specialist — SRA-007
- └─> Instructor Certification — SRA-009
 - └─> Faculty Position
 - └─> SRA Mentor

PART TWO: PROPRIETARY ACADEMY STANDARDS

“These ten standards are the non-negotiable foundation of every SRA course. They define what it means to be SRA-certified. They are our proprietary quality architecture.”

2.1 SRA Teaching Standard (SRA-TS-001)

Purpose

To ensure every SRA course is taught with pedagogical excellence, Islamic educational values, and adult-learning best practice.

Standard Requirements

1. Qualified Faculty

- Every instructor must hold the SRA-009 Instructor Certification OR equivalent verified teaching qualification
- Subject matter expertise: minimum 3 years clinical practice for clinical modules; verified Islamic scholarship for Islamic modules
- Annual faculty development: minimum 10 hours CPD in teaching methodology

2. Evidence-Based Pedagogy

- All lessons designed using Knowles' Andragogy principles
- Active learning: minimum 60% of lesson time must be interactive (not lecture-only)
- Formative assessment: every lesson must include a knowledge check
- Spaced repetition: key concepts revisited across multiple lessons

3. Islamic Educational Values

- Every lesson opens with the intention (niyyah) for seeking beneficial knowledge
- Islamic integration is natural, not forced — relevant to content, not decorative
- Respect for the sanctity of knowledge (talab al-'ilm) in all interactions

- Humility in teaching: instructors model intellectual honesty

4. Differentiated Instruction

- Content adapted for mixed-ability cohorts
- Additional support for students without healthcare backgrounds
- Extension activities for students with prior clinical training
- Accessibility compliance: all materials meet WCAG 2.1 AA standards

5. Quality Metrics

- Student satisfaction: minimum 4.5/5.0 average
 - Knowledge retention: 80%+ on delayed post-tests (3 months)
 - Competency achievement: 90%+ pass rate on first OSCE attempt
 - Faculty observation: annual peer review of teaching
-

2.2 SRA Clinical Practice Standard (SRA-CPS-001)

Purpose

To ensure all clinical teaching and practice meets or exceeds international safety and quality standards.

Standard Requirements

1. Evidence-Informed Protocols

- All clinical protocols based on systematic review evidence where available
- Traditional protocols clearly identified as such with scholarly sourcing
- Annual review of protocols against new evidence
- Clear distinction between established practice and experimental techniques

2. Safety-First Hierarchy

- Patient safety is the non-negotiable priority in all clinical decisions
- Risk assessment is mandatory before every treatment
- Contraindication screening is never skipped, never rushed
- Emergency preparedness: every clinic must have emergency protocols and equipment

3. Infection Control

- Universal precautions applied without exception
- Single-use items are never reused
- Sterilisation hierarchy: critical (sterile), semi-critical (high-level disinfection), non-critical (low-level disinfection)
- Environmental cleaning: between patients, daily deep clean, weekly audit
- Waste management: segregated, labelled, licensed disposal

4. Documentation Standards

- Every patient contact documented contemporaneously
- Consent documented before every treatment
- Standardised record format across all SRA-affiliated clinics
- Records retained for minimum 7 years (or per local regulation)
- Photography: consent mandatory, storage secure, access controlled

5. Equipment Standards

- All equipment CE marked or FDA cleared where applicable
 - Regular calibration and maintenance of all reusable equipment
 - Disposable items from approved suppliers only
 - Equipment audit: annual inventory and compliance check
-

2.3 SRA Assessment Standard (SRA-AS-001)

Purpose

To ensure assessment is rigorous, fair, valid, reliable, and aligned with learning outcomes.

Standard Requirements

1. Validity

- Every assessment item maps to specific learning outcomes
- Assessment measures what it claims to measure
- Practical assessments use authentic clinical scenarios
- Written assessments test application, not just recall

2. Reliability

- Standardised marking rubrics for all assessments
- Double-marking for borderline cases
- Inter-rater reliability checks: minimum 90% agreement
- Clear, published criteria for all assessments

3. Fairness

- Reasonable adjustments for students with disabilities
- Clear appeals process published in student handbook
- No hidden assessments or surprise requirements
- Feedback provided for all assessments, not just marks

4. Rigor

- Pass mark: 70% (reflecting professional standards)

- Must-pass components: all OSCE stations must be passed individually
- No compensation between domains: clinical competence cannot compensate for poor safety knowledge
- Re-sit policy: one free re-sit within 6 months; second re-sit at administrative cost

5. Authenticity

- OSCE stations simulate real clinical scenarios
- Portfolio requires actual supervised practice cases
- Case studies use realistic patient presentations
- Assessment conditions mirror professional practice

2.4 SRA Professional Conduct Standard (SRA-PCS-001)

Purpose

To define the standards of professional behaviour expected of all SRA students, graduates, and faculty.

Standard Requirements

1. Scope of Practice

- Practitioners work within their certified competence
- Clear understanding of what Hijama practitioners can and cannot do
- Appropriate referral to other healthcare professionals when indicated
- No practice beyond certification level without additional training

2. Patient Relationships

- Professional boundaries maintained at all times
- No exploitation of the therapeutic relationship
- Respect for patient autonomy and dignity
- Cultural sensitivity in all interactions

3. Confidentiality

- Patient information shared only with consent or legal obligation
- Digital records encrypted and access-controlled
- Discussion of cases anonymised in teaching and research
- GDPR compliance (or local equivalent)

4. Marketing Ethics

- No unsubstantiated therapeutic claims
- No disparagement of conventional medicine
- Honest representation of qualifications and experience

- Clear pricing with no hidden costs

5. Interprofessional Conduct

- Respectful collaboration with other healthcare professionals
- Appropriate referral and communication with GPs and specialists
- No advice that conflicts with patient's existing medical care
- Recognition of Hijama as complementary, not alternative, to conventional care

2.5 SRA Competency Framework (SRA-CF-001)

Purpose

To define the competencies required for SRA certification across all domains of practice.

The Six Competency Domains

Domain	Level 1: Foundational	Level 2: Developing	Level 3: Competent	Level 4: Proficient	Level 5: Expert
Knowledge (Ilm)	Recalls facts	Explains concepts	Applies knowledge	Synthesises and teaches	Contributes to knowledge
Clinical Assessment (Tashkhis)	Identifies components	Performs with guidance	Performs independently	Adapts to complex cases	Develops new approaches
Technical Skills (Itqan)	Observes and describes	Practises with supervision	Performs competently	Refines and innovates	Masters and teaches
Safety Management (Hifz)	Recognises hazards	Applies protocols	Manages incidents	Prevents and educates	Leads safety culture
Communication (Kalam)	Listens actively	Communicates clearly	Adapts to context	Inspires and mentors	Influences professionally
Professionalism (Amanah)	Understands ethics	Applies principles	Models excellence	Leads and influences	Shapes the profession

Competency Assessment

- Competency assessed at multiple points: formative (during course), summative (OSCE), and longitudinal (portfolio)
- Minimum Level 3 (Competent) required for certification in all domains
- Level 4 (Proficient) expected for SRA-002 Advanced certification
- Level 5 (Expert) required for SRA-009 Instructor Certification

2.6 SRA Graduate Attributes (SRA-GA-001)

Purpose

To define the attributes every SRA graduate will demonstrate — beyond technical competence.

The Eight Graduate Attributes

GA-1: Islamic Intellectual Formation

- Sound understanding of the Islamic foundations of Prophetic Medicine
- Ability to navigate between revelation, scholarly tradition, and scientific evidence
- Commitment to the spiritual dimensions of healing practice

GA-2: Clinical Reasoning

- Systematic approach to patient assessment and treatment planning
- Evidence-informed decision-making with awareness of uncertainty
- Recognition of complexity and comfort with clinical ambiguity

GA-3: Technical Mastery

- Precise, safe, effective procedural skills
- Attention to detail in every aspect of practice
- Continuous refinement of technique through reflection and feedback

GA-4: Patient Safety Advocacy

- Uncompromising commitment to patient safety
- Proactive risk identification and management
- Emergency preparedness and response competence

GA-5: Communication Excellence

- Clear, compassionate, culturally sensitive patient communication
- Honest communication about evidence, risks, and benefits
- Professional interprofessional communication

GA-6: Ethical Practice

- Integrity in every clinical and business decision
- Respect for patient autonomy, dignity, and confidentiality
- Practice within scope and competence

GA-7: Lifelong Learning

- Commitment to continuous professional development
- Critical engagement with new evidence and evolving practice
- Intellectual humility and openness to learning

GA-8: Community Contribution

- Commitment to serving the community through healing
 - Willingness to mentor and support other practitioners
 - Contribution to the advancement of the profession
-

2.7 SRA Code of Ethics (SRA-CE-001)

Purpose

To provide an ethical framework grounded in both Islamic principles and professional healthcare ethics.

The Five Pillars of SRA Ethics

Pillar 1: Preservation of Life (Hifz al-Nafs)

- Patient safety is the paramount consideration
- No treatment performed when risk exceeds likely benefit
- Emergency preparedness and appropriate referral
- Continuous competence maintenance

Pillar 2: Preservation of Intellect (Hifz al-'Aql)

- Honest communication of evidence — never misleading patients
- No unsubstantiated claims about treatment efficacy
- Critical appraisal of one's own practice and limitations
- Rejection of pseudoscience and anti-intellectualism

Pillar 3: Preservation of Religion (Hifz al-Din)

- Respect for patients of all faiths and none
- No coercion or pressure regarding religious practice
- Honest representation of Islamic scholarly positions
- Rejection of superstition presented as religion

Pillar 4: Preservation of Lineage (Hifz al-Nasl)

- Respect for family structures and relationships
- Appropriate chaperone policies and gender-sensitive practice
- Protection of reproductive health and fertility
- Consideration of familial implications of treatment

Pillar 5: Preservation of Property (Hifz al-Mal)

- Fair and transparent pricing
- No exploitation of patient vulnerability
- Efficient use of resources
- Honest business practices

Ethical Decision-Making Framework

When facing an ethical dilemma, SRA practitioners apply the **RAJA Framework**:

- **Reflect:** Pause and identify the ethical dimensions
 - **Assess:** Consider all stakeholders and the Maqasid al-Shari'ah
 - **Judge:** Apply the Code of Ethics and seek counsel if needed
 - **Act:** Decide and document with clear ethical reasoning
-

2.8 SRA Clinical Excellence Framework (SRA-CEF-001)

Purpose

To define what clinical excellence means at SRA — going beyond competence to mastery.

The Six Dimensions of Clinical Excellence

1. Knowledge Depth

- Comprehensive understanding of anatomy, physiology, and pathology relevant to practice
- Current awareness of evidence base with critical appraisal
- Understanding of Islamic scholarly tradition and its clinical application

2. Technical Precision

- Procedural skills performed with consistency and precision
- Adaptation of technique to individual patient needs
- Minimisation of patient discomfort through skill

3. Safety Discipline

- Rigorous adherence to infection control and safety protocols
- Proactive risk management in every clinical encounter
- Emergency preparedness and response capability

4. Communication Mastery

- Patient education that is clear, honest, and empowering
- Management of expectations with compassion
- Interprofessional communication that builds collaboration

5. Ethical Integrity

- Uncompromising adherence to the Code of Ethics
- Transparency in all clinical and business dealings
- Recognition and management of conflicts of interest

6. Continuous Improvement

- Regular self-assessment and reflective practice

- Engagement with CPD and evidence updates
- Contribution to the profession through mentorship or research

Clinical Excellence Assessment

- Self-assessment against the framework: annual
- Peer review: every 2 years
- Patient feedback: continuous
- CPD portfolio: reviewed at recertification

2.9 SRA Risk Management & Patient Safety Standard (SRA-RMPS-001)

Purpose

To establish a systematic approach to risk management and patient safety that exceeds regulatory minimums.

Standard Requirements

1. Risk Assessment Framework

- Pre-treatment risk assessment: mandatory for every patient
- Risk stratification: low, moderate, high categories with corresponding protocols
- Documentation: all risk assessments recorded in patient records
- Review: risk assessment updated at every visit

2. Incident Management

- Incident reporting: all adverse events reported within 24 hours
- Near-miss reporting: encouraged and non-punitive
- Investigation: root cause analysis for all significant incidents
- Learning: incidents shared (anonymised) for organisational learning

3. Emergency Preparedness

- Emergency equipment: all clinics equipped with basic emergency kit
- Training: annual emergency response training for all practitioners
- Protocols: syncope management, excessive bleeding, anaphylaxis recognition
- Communication: clear emergency contact procedures

4. Clinical Governance

- Clinical audit: quarterly review of practice against standards
- Peer review: regular case discussion and feedback
- Continuing competence: verified through CPD and reassessment
- Scope monitoring: ensuring practice remains within competence

5. Safeguarding

- Safeguarding training: all practitioners trained in vulnerability recognition
- Policies: clear policies for vulnerable adults and children
- Reporting: procedures for raising safeguarding concerns
- Record-keeping: meticulous documentation of safeguarding issues

2.10 SRA Continuing Professional Development Standard (SRA-CPD-001)

Purpose

To ensure SRA-certified practitioners maintain and enhance their competence throughout their careers.

CPD Requirements

Cycle: 2 years **Total Hours:** 40 hours minimum

Category	Hours	Examples
Clinical Practice	15 hours	Supervised practice; case review; peer observation; clinical audit
Education	10 hours	SRA workshops; webinars; conference attendance; journal clubs
Research Literacy	8 hours	Journal review; evidence updates; research participation; critical appraisal
Islamic Studies	5 hours	Relevant courses; scholarly lectures; personal study with verification
Professional Development	2 hours	Business skills; communication training; ethics updates

CPD Recording

- Digital CPD portfolio maintained on SRA Community platform
- Evidence required for all CPD activities (certificates, reflections, logs)
- Annual self-assessment against Competency Framework
- Peer verification for clinical practice hours

Recertification

- CPD portfolio submitted for review at end of 2-year cycle
- Portfolio reviewed by SRA Academic Board
- Deficiencies identified with support plan provided
- Certification suspended if requirements not met within grace period
- Re-activation pathway available

PART THREE: SIGNATURE LEARNING FRAMEWORKS

“These eight frameworks are proprietary intellectual property of Sunnah Remedies Academy. They appear throughout every SRA course and define the SRA approach to clinical practice, learning, and professional development.”

3.1 The SAFE Clinical Protocol

Systematic | Anatomical | Faithful | Evidence-informed

Purpose

A four-pillar framework that governs every clinical treatment performed by SRA graduates. It is the signature clinical decision-making tool of the academy.

The Four Pillars

S — Systematic Every treatment follows a structured, repeatable process:

1. Pre-treatment assessment (7-Step Assessment)
2. Risk stratification and contraindication screening
3. Informed consent
4. Treatment planning (point selection, parameter setting)
5. Procedure execution (step-by-step protocol)
6. Monitoring and response assessment
7. Aftercare and follow-up planning

Rationale: Systematic practice reduces error, ensures consistency, and creates documentation trails. The Prophet (peace be upon him) said: “Allah loves that when one of you does something, he does it with excellence” (Sahih). Systematic practice is ihsan in clinical form.

A — Anatomical Every cup placement is based on sound anatomical knowledge:

- Surface anatomy landmarks identified by palpation
- Underlying structures (vessels, nerves, organs) mapped and avoided
- Muscle groups, fascial planes, and trigger points considered
- Individual anatomical variation assessed

Rationale: Anatomical knowledge is the non-negotiable foundation of safe practice. The human body is an amanah — it must be treated with the respect that comes from deep knowledge.

F — Faithful Every treatment honours the Sunnah while practising within evidence-informed boundaries:

- Sunnah points incorporated where clinically appropriate
- Islamic ethical principles guide every patient interaction

- The spiritual dimension of healing is acknowledged and respected
- Scholarly tradition informs practice where evidence is limited

Rationale: SRA graduates are practitioners of Prophetic Medicine. The faith dimension is not decorative — it is integral to the therapeutic relationship and the practitioner’s intention.

E — Evidence-informed Every treatment integrates the best available evidence:

- Current research informs treatment parameters and point selection
- Honest communication with patients about evidence strengths and limitations
- Traditional practice respected where evidence is limited, with clear communication
- Continuous updating as new evidence emerges

Rationale: Evidence-informed practice is an Islamic ethical obligation. The Qur’an commands verification: “*O you who have believed, if there comes to you a disobedient one with information, investigate*” (49:6). This applies to medical claims as it does to all claims.

Application

The SAFE Protocol appears:

- On the wall of every SRA-affiliated clinic
- In every treatment record as a checklist
- In the practical handbook as the governing framework
- In assessment: OSCE stations evaluate SAFE compliance
- In CPD: practitioners reflect on their SAFE practice annually

3.2 The 7-Step Patient Assessment Framework

Purpose

A comprehensive, systematic patient assessment tool that ensures no critical information is missed before treatment.

The Seven Steps

Step 1: Presenting Complaint and Goals

- Open question: “What brings you here today?”
- Patient’s own goals for treatment identified
- Expectations explored and managed
- Previous experience with Hijama or other therapies

Step 2: Medical History (Systematic Review)

- Cardiovascular: hypertension, heart disease, arrhythmias, anticoagulants
- Respiratory: asthma, COPD, recurrent infections

- Endocrine: diabetes, thyroid disorders
- Neurological: epilepsy, migraines, neuropathy
- Musculoskeletal: chronic pain, arthritis, injuries
- Gastrointestinal: ulcers, liver disease, IBS
- Genitourinary: pregnancy, menstrual history, kidney disease
- Immunological: autoimmune conditions, immunosuppression
- Dermatological: skin conditions, scarring history, skin fragility
- Psychological: anxiety, depression, trauma history
- Surgical history: previous operations, implants

Step 3: Current Medications and Allergies

- All prescription medications documented
- Over-the-counter and herbal supplements noted
- Allergies: medication, latex, antiseptics, adhesives
- Blood thinners specifically flagged for risk assessment

Step 4: Previous Hijama Experience

- Previous treatments: when, where, practitioner, response
- Any adverse events or complications
- What worked and what didn't
- Current interval since last treatment

Step 5: Lifestyle Factors

- Occupation and physical demands
- Exercise habits
- Diet and hydration
- Sleep quality
- Stress levels
- Smoking and alcohol

Step 6: Vital Signs

- Blood pressure (mandatory)
- Heart rate
- Temperature (if indicated)
- Blood glucose (for diabetic patients)
- Oxygen saturation (if indicated)

Step 7: Risk Stratification and Contraindication Screening

- Absolute contraindications identified and documented
- Relative contraindications assessed and managed
- Risk category assigned: low, moderate, high
- Decision: proceed, modify, or refer
- Clinical reasoning documented

Documentation

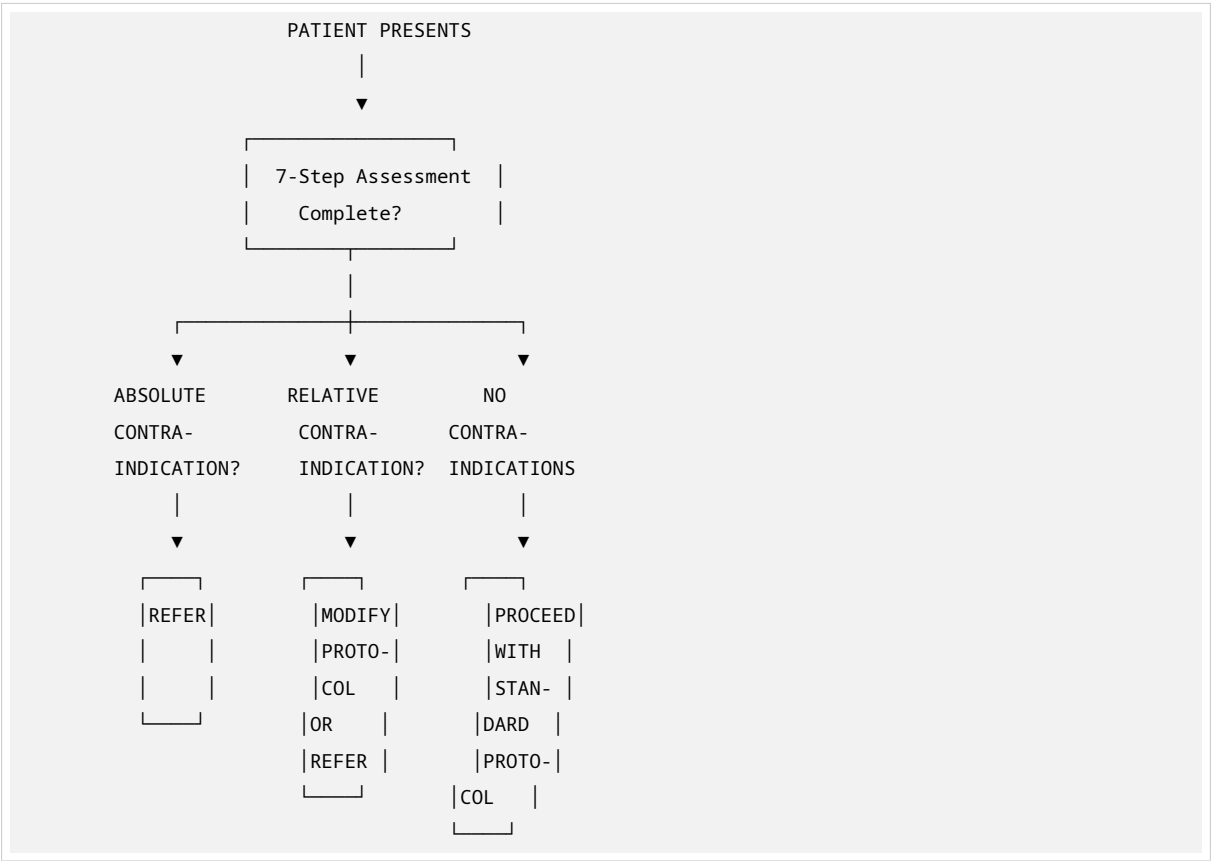
The 7-Step Assessment is documented on the SRA Patient Assessment Form. Every step must be completed and signed before treatment proceeds.

3.3 The Clinical Decision Tree

Purpose

A visual decision-support tool for determining whether to proceed with treatment, modify the protocol, or refer.

Decision Flow



Absolute Contraindications (Do Not Treat)

- Haemophilia or other bleeding disorders
- Severe anaemia (Hb < 10 g/dL)

- Active infection or fever
- Suspected deep vein thrombosis
- Pregnancy (absolute for abdominal/back points)
- Patient refusal or inability to consent

Relative Contraindications (Modify or Refer)

- Diabetes (glucose monitoring required)
 - Hypertension (BP threshold: >160/100 refer)
 - Epilepsy (seizure risk assessment)
 - Anticoagulant therapy (risk-benefit assessment)
 - Immunocompromise (enhanced precautions)
 - Elderly patients (reduced parameters)
 - Children 16-18 (guardian consent, reduced parameters)
-

3.4 The Consultation Framework

Purpose

A structured approach to the initial consultation that builds rapport, gathers information, and establishes the therapeutic relationship.

The Five Phases

Phase 1: Welcome and Rapport (5 minutes)

- Warm greeting and introduction
- Clinic environment: privacy, comfort, professionalism
- Establishing the patient's preferred communication style
- Explaining the consultation process

Phase 2: Information Gathering (15-20 minutes)

- 7-Step Patient Assessment
- Active listening and clarifying questions
- Non-verbal communication awareness
- Cultural sensitivity and gender considerations

Phase 3: Explanation and Education (10-15 minutes)

- Explaining findings in accessible language
- Discussing Hijama: what it is, what to expect, potential benefits
- Honest discussion of evidence: what is known, what is uncertain
- Answering questions thoroughly

Phase 4: Shared Decision-Making (5-10 minutes)

- Presenting options: proceed, modify, defer, refer

- Discussing risks and benefits
- Obtaining informed consent
- Setting realistic expectations

Phase 5: Planning and Closure (5 minutes)

- Treatment plan agreed
 - First appointment scheduled (if proceeding)
 - Pre-treatment instructions provided
 - Aftercare information previewed
 - Contact information and emergency procedures explained
-

3.5 The Reflect–Learn–Apply Model

Purpose

The signature learning methodology of SRA — a cyclical model that ensures deep learning and practical application.

The Three Phases

Reflect

- Before learning: assess current knowledge and identify gaps
- During learning: pause at key points to check understanding
- After learning: structured reflection on what was learned and its implications
- Islamic dimension: reflection (tafakkur) as a spiritual practice — *“Indeed, in the creation of the heavens and the earth are signs for those who reflect”* (Qur’an 3:190)

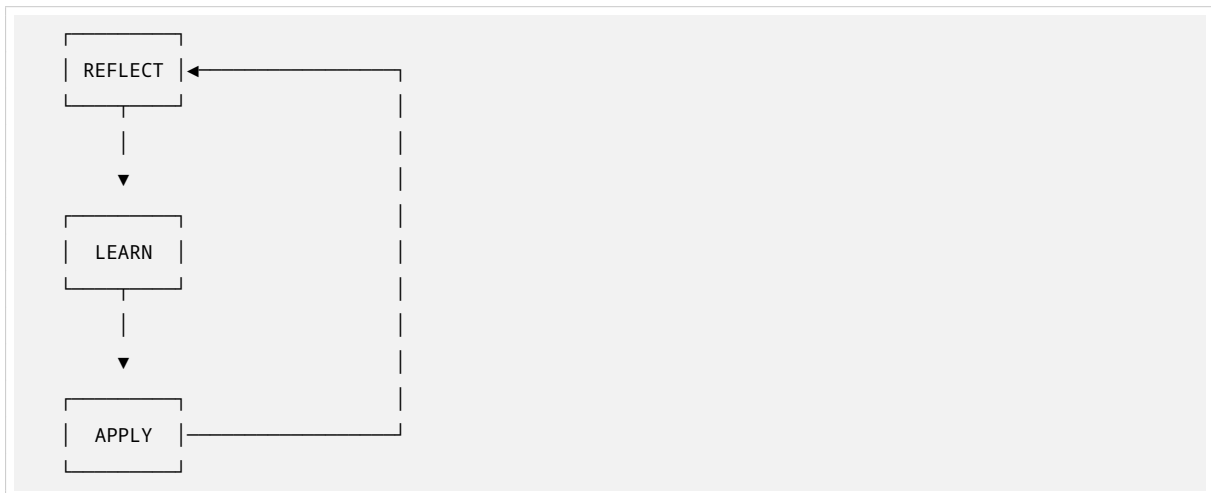
Learn

- Active engagement with material: not passive consumption
- Multiple modalities: visual, auditory, kinaesthetic, reading/writing
- Connection to prior knowledge and experience
- Critical questioning: “Why is this true?” “What is the evidence?” “How does this connect to what I already know?”

Apply

- Practical application in supervised settings
- Case study analysis
- Peer teaching and explanation
- Clinical practice with feedback
- Reflection on application: what worked, what didn’t, what to adjust

The Cycle



Every SRA lesson incorporates all three phases. The Learning Journal provided to every student is structured around this model.

3.6 The Healing Pathway

Purpose

A patient-centred framework that conceptualises the therapeutic journey from initial consultation through to wellness maintenance.

The Five Stages

Stage 1: Assessment and Understanding

- Comprehensive evaluation of the patient's condition
- Education about their health status
- Setting realistic, shared goals
- Establishing baseline measurements

Stage 2: Initial Treatment

- First Hijama session with careful monitoring
- Conservative parameters for first treatment
- Detailed observation and documentation
- Immediate response assessment

Stage 3: Active Treatment Phase

- Series of treatments at appropriate intervals
- Progress monitoring at each session
- Parameter adjustment based on response
- Integration with other healthcare as appropriate

Stage 4: Transition and Maintenance

- Reduction in treatment frequency
- Patient education for self-care
- Lifestyle recommendations (Prophetic remedies, nutrition)
- Monitoring for sustained improvement

Stage 5: Wellness and Prevention

- Maintenance Hijama at appropriate intervals
- Ongoing lifestyle support
- Early intervention if symptoms recur
- Integration of Hijama into holistic health maintenance

Practitioner Application

The Healing Pathway helps practitioners:

- Set realistic expectations with patients
- Plan treatment courses systematically
- Recognise when to transition between stages
- Identify when referral is needed
- Document progress meaningfully

3.7 The Evidence & Sunnah Matrix

Purpose

A decision-support tool that maps every clinical claim across two dimensions — evidence strength and Sunnah basis — enabling honest, transparent communication.

The Matrix

	Strong Evidence	Moderate Evidence	Preliminary Evidence	Traditional Practice
Explicit Hadith	Gold Standard: Strong evidence + Explicit Sunnah	Promising: Moderate evidence + Explicit Sunnah	Emerging: Early evidence + Explicit Sunnah	Faithful: Traditional practice + Explicit Sunnah
Scholarly Recommendation	Robust: Strong evidence + Scholarly support	Supported: Moderate evidence + Scholarly support	Developing: Early evidence + Scholarly support	Respected: Traditional practice + Scholarly support
Traditional Practice	Validated: Strong evidence + Traditional use	Plausible: Moderate evidence + Traditional use	Exploratory: Early evidence + Traditional use	Heritage: Traditional practice alone

Table 16 – continued

	Strong Evidence	Moderate Evidence	Preliminary Evidence	Traditional Practice
Clinical Experience	Anecdotal-strong: Strong anecdotal reports	Anecdotal-moderate: Moderate anecdotal reports	Anecdotal-limited: Limited reports	Novel: New application

Communication Guide

Gold Standard: “There is strong research evidence for this, and it is also supported by authentic Hadith. We can be confident in recommending this.”

Promising: “The research is promising and growing, and this practice has explicit Prophetic support. It is reasonable to offer this with appropriate expectations.”

Emerging: “Early research shows potential, and this is established in the Sunnah. We can offer this with the understanding that the evidence is still developing.”

Faithful: “This is established in authentic Hadith and has been practised for centuries. While modern research is limited, the Prophetic basis gives us confidence.”

Validated: “Strong research supports this traditional practice, even though there is no specific Hadith about it.”

Heritage: “This has been practised traditionally. We should be transparent that both specific Hadith evidence and modern research are limited.”

Application

The Evidence & Sunnah Matrix appears:

- In every module where clinical claims are made
- As a reference tool in the student handbook
- In patient communication training
- In assessment: students must correctly classify claims using the matrix

3.8 The Ihsan Quality Cycle

Purpose

A continuous quality improvement framework that ensures SRA programmes and practitioners are always improving.

The Four Stages

Plan

- Set quality standards (based on this Framework)
- Identify quality indicators and metrics

- Establish targets and timelines
- Allocate resources for quality activities

Do

- Deliver teaching and clinical services to standard
- Collect data on quality indicators
- Document processes and outcomes
- Maintain records for analysis

Check

- Analyse quality data against targets
- Identify gaps and variances
- Gather feedback from students, patients, and faculty
- Compare against benchmarks

Act

- Implement improvements based on findings
- Update standards and protocols
- Communicate changes to all stakeholders
- Begin the cycle again

Quality Indicators

- Student satisfaction scores
- Knowledge retention rates (delayed testing)
- OSCE pass rates and first-attempt rates
- Graduate employment/practice rates
- Patient satisfaction (graduate practice surveys)
- Incident reports and near-misses
- CPD compliance rates
- Faculty development participation

PART FOUR: CURRICULUM ARCHITECTURE — SRA-001

4.1 Programme Overview

Attribute	Specification
Course Code	SRA-001
Course Title	Certified Hijama Practitioner
Award	CHP (Certified Hijama Practitioner)

Table 17 – continued

Attribute	Specification
Accreditation	CMA & IPHM Accredited
Total Hours	120 guided learning hours + 40 supervised practice hours
Duration	12 weeks (part-time) or 4 weeks (intensive)
Delivery	Blended: live online lectures + in-person practical workshops + self-directed study
Assessment	Written examination (40%) + Practical OSCE (40%) + Portfolio (20%)
Prerequisites	Minimum age 18; basic literacy in English; no prior clinical qualification required
Maximum Class Size	16 students (to ensure individual practical attention)
Certification Validity	2 years (renewable via CPD)

4.2 Learning Outcomes

Domain 1: Knowledge (K)

K1. Explain the historical development of cupping therapy across civilisations with scholarly accuracy, from ancient Egypt through the Islamic Golden Age to contemporary revival.

K2. Articulate the Islamic foundations of Hijama with proper referencing of Qur'an (with Arabic text where relevant), authentic Hadith (with full isnad and grading), and classical scholarly commentary.

K3. Describe the anatomy and physiology relevant to cupping practice with clinical precision — skin, circulatory system, lymphatic system, fascia, musculature, and peripheral nerves.

K4. Summarise the current evidence base for cupping therapy, including strengths, limitations, and gaps, using GRADE criteria and critical appraisal methodology.

K5. Explain the mechanisms of action proposed for cupping therapy, distinguishing between established (nitric oxide, myofascial decompression), probable (immune modulation), and theoretical (Taibah Theory) pathways.

K6. Identify contraindications, precautions, and risk factors for cupping therapy, applying the Clinical Decision Tree to individual patient presentations.

K7. Describe infection prevention and control standards applicable to wet cupping practice, including the sterilisation hierarchy, waste management, and environmental cleaning.

K8. Explain the legal and regulatory framework governing complementary therapy practice in key jurisdictions (UK, USA, Canada, Australia, UAE, KSA).

Domain 2: Clinical Skills (S)

S1. Conduct a comprehensive pre-treatment patient assessment using the 7-Step Patient Assessment Framework, including history, examination, vital signs, and risk stratification.

- S2.** Obtain valid informed consent through clear communication of benefits, risks, alternatives, and the right to refuse — documented using the SRA Consent Form.
- S3.** Perform wet cupping safely using correct technique, equipment, and infection control measures — following the SAFE Protocol step-by-step.
- S4.** Apply appropriate cup placement based on clinical reasoning, anatomical knowledge, Sunnah points, and patient presentation.
- S5.** Manage common adverse events (excessive bleeding, syncope, minor infection) and recognise when emergency intervention is required.
- S6.** Provide comprehensive aftercare advice using the SRA Aftercare Protocol — immediate, 24-hour, days 2-10, and follow-up phases.
- S7.** Maintain accurate clinical documentation in compliance with legal and professional requirements using SRA-standardised forms.

Domain 3: Professional Practice (P)

- P1.** Practice within scope of competence and recognise when referral to other healthcare professionals is appropriate — applying the Clinical Decision Tree.
- P2.** Apply ethical principles derived from both Islamic teaching (Maqasid al-Shari'ah) and professional healthcare standards — following the SRA Code of Ethics.
- P3.** Communicate effectively with patients from diverse backgrounds, including sensitive discussion of Islamic dimensions, using the Consultation Framework.
- P4.** Maintain professional indemnity insurance and comply with all legal requirements for practice in their jurisdiction.
- P5.** Commit to continuing professional development and evidence-based practice — understanding the CPD requirements for certification maintenance.
- P6.** Operate a Hijama clinic or practice within an existing healthcare setting with business competence — including ethical marketing, pricing, and patient retention.

Domain 4: Islamic Integration (I)

- I1.** Distinguish accurately between Qur'anic teaching, authentic Sunnah (Sahih, Hasan), scholarly interpretation (ijtihad), traditional practice (taqlid), and scientific evidence — using the Evidence & Sunnah Matrix.
- I2.** Integrate Islamic ethical principles (Maqasid al-Shari'ah) naturally into clinical practice — not as an add-on but as a foundational framework.
- I3.** Communicate the Islamic dimensions of Hijama with accuracy, respect, and appropriate context — adapting to patients of all faiths and none.
- I4.** Demonstrate personal commitment to the spiritual dimensions of healing practice — recognising Allah as the ultimate Healer (al-Shafi) while fulfilling the duty to seek and apply means.

4.3 Curriculum Map — 17-Module Structure

The SRA-001 curriculum is organised into 17 modules, mapped directly to the user’s specification and the four learning domains. This structure ensures comprehensive coverage while maintaining logical pedagogical progression.

Module	Title	Hours	Lessons	Learning Domains	Key Frameworks
M1	Introduction to Hijama	8	2	K1, K2, I1, I2	Evidence & Sunnah Matrix
M2	History of Cupping Around the World	8	2	K1, I1	Historical Timeline
M3	Hijama in Islam	12	3	K2, I1, I2, I3	Evidence & Sunnah Matrix
M4	Evidence-Based Medicine & Research	12	3	K4, K5, I1	Evidence & Sunnah Matrix
M5	Human Anatomy for Practitioners	16	4	K3, S4	7-Step Assessment
M6	Equipment, Instruments & Supplies	8	2	K7, S3	SAFE Protocol
M7	Infection Prevention & Control	8	2	K7, S3	Infection Control Standard
M8	Patient Assessment & Clinical Reasoning	12	3	S1, S2, P1, P2	7-Step Assessment, Clinical Decision Tree
M9	Dry Cupping: Theory & Technique	8	2	S3, S4	SAFE Protocol
M10	Wet Cupping (Hijama): Procedure	16	4	S3, S4, S5, S6	SAFE Protocol, Aftercare Protocol
M11	Hijama Points: Traditional & Clinical	12	3	S4, K2, I2	SAFE Protocol
M12	Conditions & Applications	8	2	K4, S4, P1	Evidence & Sunnah Matrix, Clinical Decision Tree
M13	Complications & Emergency Management	8	2	S5, K6, P1	Aftercare Protocol, Risk Management Standard

Table 18 – continued

Module	Title	Hours	Lessons	Learning Domains	Key Frameworks
M14	Clinical Communication & Professionalism	8	2	P2, P3, I3	Consultation Framework, Code of Ethics
M15	Building Your Practice	8	2	P4, P5, P6	Clinical Excellence Framework
M16	Practical Assessment (OSCE Preparation)	8	2	S1-S7, P1-P3	All Frameworks
M17	Final Examination & Certification	4	1	All Domains	All Frameworks
Practical Workshops	Supervised Clinical Practice	40	10	S1-S7, P1-P6	All Clinical Frameworks
Assessment	Written Exam + OSCE + Portfolio	8	—	All Domains	Assessment Standard

Total: 204 hours (120 guided learning + 40 supervised practice + 44 self-directed study)

Module Sequencing Rationale

The curriculum follows a deliberate pedagogical progression:

Phase 1: Foundations (Modules 1-4) — Why Hijama matters

- M1: Introduction — establishes purpose, motivation, and scope
- M2: History — builds scholarly context and intellectual depth
- M3: Islamic foundations — grounds practice in authentic tradition
- M4: Evidence — develops critical appraisal and evidence literacy

Phase 2: Knowledge (Modules 5-7) — What the body looks like and how to protect it

- M5: Anatomy — develops anatomical knowledge essential for safe practice
- M6: Equipment — ensures proper tool selection and use
- M7: Infection control — establishes safety as the non-negotiable foundation

Phase 3: Skills (Modules 8-11) — How to assess and treat

- M8: Patient assessment — systematic assessment before treatment
- M9: Dry cupping — foundational technique before wet cupping
- M10: Wet cupping — the complete procedure with precision

- M11: Points — anatomical and traditional point selection

Phase 4: Application (Modules 12-15) — How to apply knowledge and skills

- M12: Conditions — evidence-informed treatment planning
- M13: Complications — safety, emergency management, aftercare
- M14: Communication — professionalism and patient relationships
- M15: Practice — business, ethics, and professional development

Phase 5: Assessment (Modules 16-17) — Demonstrating competence

- M16: OSCE preparation — consolidating practical skills
- M17: Final examination — comprehensive assessment

4.4 Detailed Module Specifications

MODULE 1: Introduction to Hijama

8 hours | 2 lessons

Lesson 1.1 — What is Hijama? Definitions, Scope, and Identity (4 hours)

- Learning Objectives:
 - Define wet cupping (Hijama) with clinical precision
 - Distinguish between wet cupping, dry cupping, and other modalities
 - Explain the regulatory status of cupping therapy in key jurisdictions
 - Articulate the professional identity of a Hijama practitioner
- Content:
 - Definition: controlled incision + negative pressure + blood extraction
 - Modalities: dry cupping, fire cupping, massage cupping, flash cupping, herbal cupping, needle cupping
 - Regulatory landscape: UK, USA, Canada, Australia, UAE, KSA
 - Scope of practice: what CHPs can and cannot do
 - Professional associations and accreditation bodies
 - The CHP credential: what it signifies
- Signature Elements:
 - SRA Clinical Pearl: The Hijama Definition Framework
 - SRA Comparison Table: All cupping modalities
 - SRA Practitioner Tip: Explaining the difference to patients
 - Reflect–Learn–Apply: Professional identity reflection

Lesson 1.2 — The Hijama Practitioner: Role, Responsibility, and Excellence (4 hours)

- Learning Objectives:
 - Understand the ethical framework governing practice
 - Apply the Competency Framework to self-assessment
 - Develop a personal professional development plan
 - Content:
 - The practitioner as amanah-holder (trustee)
 - Ihsan in clinical practice: excellence as worship
 - Scope of practice and professional boundaries
 - Interprofessional relationships and appropriate referral
 - Personal health and wellbeing for practitioners
 - Building a continuous learning mindset
 - Signature Elements:
 - SRA Islamic Insight: The Ethics of Healing — *“And We send down of the Qur’an that which is healing and mercy for the believers”* (Qur’an 17:82)
 - SRA Professional Standards: The Amanah Framework
 - SRA Reflect & Apply: Personal development planning
 - Reflect–Learn–Apply: Intention-setting exercise
-

MODULE 2: History of Cupping Around the World

8 hours | 2 lessons

Lesson 2.1 — Ancient Civilisations and the Evolution of Cupping (4 hours)

- Learning Objectives:
- Trace the documented history of cupping from ancient Egypt to the medieval period
- Identify contributions from Egyptian, Chinese, Greek, and Islamic civilisations
- Evaluate historical sources with scholarly discrimination
- Content:
- Ancient Egypt: Ebers Papyrus (c. 1550 BCE), archaeological evidence
- Ancient China: Documentation from 281 CE, evolution of instruments
- Ancient Greece: Hippocratic corpus, Galen’s humoral theory
- The Islamic Golden Age: institutional development of cupping
- Evolution of instruments: animal horns to modern disposable systems
- Signature Elements:
- SRA Historical Timeline: Visual timeline across civilisations

- SRA Evidence Update: Archaeological findings 2020-2026
- SRA Scholarly Note: Source criticism in historical claims
- Reflect–Learn–Apply: What history teaches us about practice

Lesson 2.2 — The Islamic Golden Age to Contemporary Revival (4 hours)

- Learning Objectives:
 - Analyse the contributions of key Islamic scholars to cupping literature
 - Understand the decline and contemporary revival of Hijama
 - Evaluate the current global landscape of cupping practice
 - Content:
 - Ibn Sina (Avicenna): Al-Qanun fi al-Tibb — cupping indications and contraindications
 - Al-Zahrawi (Albucasis): surgical instruments and techniques
 - Al-Razi (Rhazes): clinical descriptions and case documentation
 - Ibn al-Qayyim al-Jawziyya: Zad al-Ma'ad — comprehensive Hijama scholarship
 - Decline during colonial period and modern revival
 - Contemporary institutional development: Sunnah Remedies and others
 - Global spread: Middle East, South Asia, Europe, Americas, Africa
 - Signature Elements:
 - SRA Historical Timeline: Islamic scholars and their contributions
 - SRA Scholarly Commentary: Ibn al-Qayyim on Hijama
 - SRA Evidence Update: Contemporary scholarly positions
 - Reflect–Learn–Apply: Our place in this tradition
-

MODULE 3: Hijama in Islam

12 hours | 3 lessons

Lesson 3.1 — Qur'anic Foundations and Authentic Hadith (4 hours)

- Learning Objectives:
 - Quote and reference Qur'anic verses related to healing and medicine
 - Cite authentic Hadith concerning Hijama with proper sourcing (isnad and grading)
 - Explain the scholarly methodology of Hadith authentication
- Content:
 - Qur'anic foundation:
 - “*And We send down of the Qur'an that which is healing and mercy for the believers*” — Qur'an 17:82 (<https://quran.com/17/82>)

- “*And when I am ill, it is He who cures me*” — Qur’an 26:80 (<https://quran.com/26/80>)
- “*O mankind, there has to come to you instruction from your Lord and healing for what is in the breasts and guidance and mercy for the believers*” — Qur’an 10:57 (<https://quran.com/10/57>)
- Authentic Hadith (Sahih):
- “The best treatment is Hijama” — Sahih al-Bukhari, Hadith 5371 (<https://sunnah.com/bukhari:5371>) — Sahih
- “If there is any good in your treatments, it is in Hijama” — Sahih al-Bukhari, Hadith 5696 (<https://sunnah.com/bukhari:5696>) — Sahih
- The Prophet (peace be upon him) had Hijama performed on his head (Al-Kahil point) — Sahih al-Bukhari, Hadith 5694 (<https://sunnah.com/bukhari:5694>) — Sahih
- The Prophet (peace be upon him) had Hijama performed on the two veins of his neck (Akhda’een) — Sahih Muslim, Hadith 5706 (<https://sunnah.com/muslim:5706>) — Sahih
- Hadith authentication methodology: Sahih, Hasan, Da’if gradings
- The six books of Hadith: Bukhari, Muslim, Abu Dawud, Tirmidhi, Nasa’i, Ibn Majah
- Signature Elements:
- SRA Islamic Insight: Hadith authentication framework with Arabic text
- SRA Scholarly Commentary: Classical positions on Hijama evidence
- SRA Evidence & Sunnah Matrix: Classification of Hijama evidence
- Reflect–Learn–Apply: Personal connection to the tradition

Lesson 3.2 — The Prophet’s Practice and Sunnah Points (4 hours)

- Learning Objectives:
- Describe the Prophet’s own practice of Hijama: locations, timing, and circumstances
- Locate the five primary Sunnah points with anatomical precision
- Apply Sunnah points within a broader clinical treatment plan
- Content:
- The Prophet’s practice:
- Locations: Al-Kahil (between shoulder blades), Akhda’een (neck veins), Al-Hamma (vertex)
- Timing: 17th, 19th, 21st of the lunar month (scholarly discussion)
- Who performed it: Abu Tayba (the Prophet’s cupper)
- The five Sunnah points:
- **Al-Kahil** (DU14/DU15 region): between the shoulder blades — the primary point
- **Akhda’een**: the two veins at the sides of the neck
- **Al-Hamma**: the vertex (top of the head)
- **Umm al-Muqeeth**: the occipital region
- **Al-Katifain** (GB21): the shoulder points

- Authentic Hadith evidence for each point with full references
- Scholarly commentary: Ibn al-Qayyim, Al-Dhahabi, contemporary scholars
- Integrating Sunnah points with clinical points
- Signature Elements:
- SRA Medical Illustration: Sunnah Points Anatomical Map
- SRA Islamic Insight: The Prophetic Basis for Each Point with Arabic text
- SRA Clinical Pearl: Sunnah Points in Modern Practice
- Reflect–Learn–Apply: Balancing tradition and evidence

Lesson 3.3 — Scholarly Tradition, Misconceptions, and Contemporary Issues (4 hours)

- Learning Objectives:
- Explain the scholarly tradition of Tibb al-Nabawi (Prophetic Medicine)
- Identify and correct common misconceptions about Hijama in Islam
- Navigate contemporary scholarly debates with intellectual honesty
- Content:
- Tibb al-Nabawi: definition, scope, and scholarly methodology
- Classical scholars on Hijama: Imam Malik, Imam Ahmad, Imam al-Nawawi, Ibn Taymiyyah
- The Isra and Mi'raj narrations: scholarly analysis of authenticity
- Sunnah days: 17th, 19th, 21st — evidence and scholarly positions
- Common misconceptions:
- “Hijama cures everything” — correcting the exaggeration
- “Hijama replaces medical treatment” — clarifying the complementary role
- “Only on Sunnah days” — scholarly flexibility
- “The blood is dirty/toxic” — scientific accuracy in communication
- Contemporary scholarly positions: Al-Azhar, Saudi Fatwa Committee, others
- Maqasid al-Shari'ah: how Hijama serves the objectives of Islamic law
- Signature Elements:
- SRA Islamic Insight: Maqasid al-Shari'ah and Healing
- SRA Scholarly Commentary: Contemporary fatwas on Hijama
- SRA Common Mistakes: Misconceptions to avoid in patient communication
- Reflect–Learn–Apply: Intellectual honesty in tradition

MODULE 4: Evidence-Based Medicine & Research

12 hours | 3 lessons

Lesson 4.1 — Research Evidence: Systematic Reviews and Critical Appraisal (4 hours)

- Learning Objectives:
- Navigate the peer-reviewed literature on cupping therapy
- Evaluate evidence quality using GRADE criteria
- Communicate evidence honestly to patients
- Content:
- Overview of major systematic reviews (2022-2026)
- Evidence by condition: back pain, neck pain, migraine, hypertension, diabetes, infertility
- Methodological limitations: small samples, heterogeneity, blinding challenges
- GRADE evidence quality ratings applied to cupping research
- How to read a scientific paper: structure, statistics, interpretation
- Communicating uncertainty: “the evidence suggests” vs. “the evidence proves”
- Signature Elements:
- SRA Evidence & Sunnah Matrix: Research evidence classification
- SRA Evidence Update: Latest systematic reviews
- SRA Clinical Pearl: Talking to patients about evidence
- Reflect–Learn–Apply: Evidence literacy as Islamic duty

Lesson 4.2 — Proposed Mechanisms of Action (4 hours)

- Learning Objectives:
- Explain the major proposed mechanisms for cupping’s therapeutic effects
- Distinguish between established, probable, and theoretical mechanisms
- Integrate mechanism understanding with clinical reasoning
- Content:
- **Established:** Nitric oxide release and reactive hyperemia; Myofascial Decompression (MFD)
- **Probable:** Immune modulation and reticuloendothelial system stimulation; Lymphatic stimulation
- **Theoretical:** Taibah Theory (Causative Pathological Substances filtration); Gate control theory
- **Emerging:** Fascial layer separation research; Connective tissue manipulation
- Placebo considerations: honest discussion of placebo effects
- Integrating mechanisms: matching mechanism to clinical presentation
- Signature Elements:
- SRA Clinical Pearl: The Mechanisms Framework

- SRA Evidence Update: Mechanism research 2024-2026
- SRA Comparison Table: Mechanisms by condition
- Reflect–Learn–Apply: Mechanism-based treatment planning

Lesson 4.3 — Blood Analysis, Dark Field Microscopy, and Scientific Literacy (4 hours)

- Learning Objectives:
 - Understand the principles of dark field microscopy as applied to cupping
 - Evaluate claims about blood analysis with critical scientific literacy
 - Communicate honestly about what blood analysis can and cannot demonstrate
 - Content:
 - Principles of dark field microscopy: technique and limitations
 - Live blood analysis: what it shows, what it doesn't show
 - Dried blood analysis: patterns and interpretation
 - Research by Dr. David Parker: findings and limitations
 - Rouleaux formation, fibrin, toxic metals: scientific context
 - The gap between observation and clinical significance
 - Ethical communication: avoiding unsubstantiated claims
 - Scientific literacy: evaluating claims critically
 - Signature Elements:
 - SRA Evidence Update: Blood analysis research
 - SRA Clinical Pearl: Communicating limitations honestly
 - SRA Common Mistakes: Overinterpreting blood findings
 - Reflect–Learn–Apply: The ethics of honest communication
-

MODULE 5: Human Anatomy for Practitioners

16 hours | 4 lessons

Lesson 5.1 — Skin Anatomy and Histology (4 hours)

- Epidermis: structure, thickness variation, barrier function
- Dermis: papillary and reticular layers, capillary networks
- Hypodermis: subcutaneous tissue, adipose layer
- Skin thickness by body site: clinical implications for incision depth
- Skin conditions: acne, eczema, psoriasis, keloid scarring, thin/fragile skin
- Age-related skin changes: implications for elderly patients
- Wound healing phases: how cupping incisions heal

- *Signature: SRA Medical Illustration — Skin Cross-Section; Scarification Depth Guide*

Lesson 5.2 — The Circulatory System and Heart (4 hours)

- Arteries, arterioles, capillaries, venules, veins: structure and function
- Capillary networks in the skin: the target of wet cupping
- Endothelial function and vascular tone
- Blood composition: cells, plasma, proteins
- Major vessels to avoid: arteries, deep veins, varicose veins
- Heart structure: chambers, valves, coronary circulation
- Cardiovascular conditions: hypertension, hypotension, arrhythmias
- Blood thinners and anticoagulants: mechanism and clinical considerations
- *Signature: SRA Medical Illustration — Superficial Vascular Anatomy; BP Monitoring Protocol*

Lesson 5.3 — The Lymphatic System (4 hours)

- Lymphatic vessels, nodes, and organs
- Lymph formation and circulation
- Major lymph node groups: cervical, axillary, inguinal
- The thoracic duct and right lymphatic duct
- Lymphatic oedema: causes and considerations
- Cupping and lymphatic stimulation: evidence and mechanism
- Areas to avoid: lymph node regions
- *Signature: SRA Medical Illustration — Lymphatic System; Lymphatic Safety Zones*

Lesson 5.4 — Fascia, Muscles, and the Musculoskeletal System (4 hours)

- Fascia: superficial, deep, visceral — structure and function
- Myofascial pain syndrome and trigger points
- Major muscles for cupping: trapezius, erector spinae, latissimus dorsi, rhomboids, gluteals
- Muscle attachments, fibre directions, and trigger point locations
- Myofascial Decompression (MFD): the science of negative pressure on fascia
- Common musculoskeletal conditions treated with cupping
- *Signature: SRA Medical Illustration — Fascial Layers and Muscle Groups; Muscle-Based Cup Placement Guide*

MODULE 6: Equipment, Instruments & Supplies

8 hours | 2 lessons

Lesson 6.1 — Cup Types, Pump Systems, and Clinical Supplies (4 hours)

- Cup types: plastic disposable, glass, silicone, bamboo — advantages and limitations
- Pump systems: manual, electric, pressure gauges
- Sterile lancets: types, gauges, single-use requirements
- Gloves, antiseptics, gauze, dressings: selection and use
- Sharps containers and clinical waste disposal
- Regulatory compliance: CE marking, FDA clearance, local requirements
- *Signature: SRA Equipment Checklist; SAFE Protocol (Equipment)*

Lesson 6.2 — Sterilisation, Clinic Setup, and Equipment Maintenance (4 hours)

- Equipment sterilisation: autoclave, chemical, disposable
- Clinic layout: clean and dirty zones, workflow design
- Environmental requirements: lighting, ventilation, temperature
- Equipment maintenance and calibration
- Stock management and ordering systems
- *Signature: SRA Clinic Setup Checklist; Cleaning SOP*

MODULE 7: Infection Prevention & Control

8 hours | 2 lessons

Lesson 7.1 — Universal Precautions and the Chain of Infection (4 hours)

- Chain of infection: source, transmission, susceptible host
- Universal precautions: hand hygiene, PPE, safe sharps handling
- Blood-borne pathogens: HIV, Hepatitis B and C, risks and precautions
- Hand hygiene: WHO 5 Moments, technique, products
- PPE selection and use: gloves, aprons, eye protection, masks
- *Signature: SRA Infection Control Standard; The Clean-to-Dirty Workflow*

Lesson 7.2 — Sterilisation, Waste Management, and Environmental Control (4 hours)

- Sterilisation hierarchy: critical, semi-critical, non-critical items
 - Environmental cleaning: surfaces, equipment, treatment room
 - Waste management: sharps, clinical waste, general waste — segregated, labelled, licensed disposal
 - Clinic hygiene audit: daily, weekly, monthly checklists
 - Incident reporting: exposure events, near-misses
 - *Signature: SRA Infection Control Standard (full); Waste Management SOP*
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MODULE 8: Patient Assessment & Clinical Reasoning

12 hours | 3 lessons

Lesson 8.1 — The 7-Step Patient Assessment (4 hours)

- Complete walkthrough of the 7-Step Patient Assessment Framework
- History-taking skills: open questions, active listening, clarifying
- Vital signs: blood pressure technique, interpretation, recording
- Red flags: unexplained weight loss, persistent fever, new neurological symptoms, suspected malignancy
- *Signature: SRA Clinical Pearl — 7-Step Patient Assessment (full); Patient Communication Box*

Lesson 8.2 — Contraindications and Risk Stratification (4 hours)

- Absolute contraindications: haemophilia, severe anaemia, active infection, suspected DVT, pregnancy, patient refusal
- Relative contraindications: diabetes, hypertension, epilepsy, immunocompromise, children, elderly, anticoagulants
- The Clinical Decision Tree: application to case scenarios
- Condition-specific considerations and parameter adjustments
- Documenting clinical reasoning
- *Signature: SRA Contraindication Decision Tree; Risk Stratification Framework*

Lesson 8.3 — Consent, Documentation, and Legal Framework (4 hours)

- Informed consent: elements, process, documentation
 - Capacity to consent: adults, children, vulnerable adults
 - GDPR and patient data protection
 - Clinical records: what to document, how long to keep
 - Photography consent: before/after images
 - Professional indemnity insurance: requirements and coverage
 - Legal liability: negligence, scope of practice, advertising standards
 - *Signature: SRA Clinical Pearl — The Consent Conversation Framework; Documentation Checklist*
-

MODULE 9: Dry Cupping — Theory & Technique

8 hours | 2 lessons

Lesson 9.1 — Dry Cupping Theory and Application (4 hours)

- Theory of negative pressure on soft tissues
- Indications for dry cupping vs. wet cupping
- Cup selection and placement for dry cupping

- Pressure parameters and duration
- Safety considerations specific to dry cupping
- *Signature: SRA Clinical Pearl — Dry Cupping Parameters; SAFE Protocol (Dry)*

Lesson 9.2 — Dry Cupping Practical (4 hours)

- Hands-on practice: cup placement, pressure application, removal
 - Technique variations: stationary, sliding, flash
 - Troubleshooting: poor suction, patient discomfort, skin reactions
 - Supervised practice with individual feedback
 - *Signature: SRA Practical Assessment Checklist — Dry Cupping*
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MODULE 10: Wet Cupping (Hijama) — Procedure

16 hours | 4 lessons

Lesson 10.1 — Preparation and Cup Placement (4 hours)

- Step 1: Preparation (consent verification, positioning, draping, skin prep)
- Step 2: Cup placement (anatomical landmarks, suction application)
- Step 3: Waiting period (3-5 minutes, skin monitoring)
- Positioning: prone, supine, side-lying — patient comfort and access
- Draping: maintaining dignity and modesty
- *Signature: SRA Clinical Procedure Visual Guide — Preparation; SAFE Protocol*

Lesson 10.2 — Scarification and Blood Collection (4 hours)

- Step 4: Cup removal (controlled release)
- Step 5: Scarification (0.5-1mm depth, 5-7mm length, 3-5 incisions)
- Scarification technique: angle, pressure, speed
- Step 6: Reapplication (moderate suction over incisions)
- Step 7: Blood collection (monitoring colour change, volume)
- Technique variations by body site
- *Signature: SRA Clinical Pearl — Scarification Technique Mastery; SRA Medical Illustration — Scarification Cross-Section*

Lesson 10.3 — Completion, Dressings, and Troubleshooting (4 hours)

- Step 8: Completion (removal, cleaning, dressing application)
- Dressing types and selection
- Troubleshooting: poor flow, excessive bleeding, patient discomfort, vasovagal response
- Technique refinement through supervised practice

- *Signature: SRA Clinical Procedure Visual Guide — Completion; Common Mistakes Reference*

Lesson 10.4 — Aftercare and Follow-Up (4 hours)

- The SRA Aftercare Protocol:
 - Immediate: dressing, observation, hydration
 - First 24 hours: activity restrictions, wound care
 - Days 2-10: mark monitoring, healing assessment
 - Follow-up: scheduling, progress review, next treatment planning
 - Written aftercare instructions
 - Follow-up phone call protocol (24-48 hours)
 - *Signature: SRA Aftercare Protocol (full); Patient Aftercare Instruction Sheet*
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MODULE 11: Hijama Points — Traditional & Clinical

12 hours | 3 lessons

Lesson 11.1 — Sunnah Points: Deep Dive (4 hours)

- Detailed anatomical location of each Sunnah point
- Palpation techniques for accurate location
- Prophetic evidence for each point with full references
- Scholarly commentary on point selection
- Integration with modern anatomical understanding
- *Signature: SRA Medical Illustration — Sunnah Points Detailed Anatomical Map; Islamic Insight with Arabic text*

Lesson 11.2 — Clinical Points: Muscle-Based and Condition-Based (4 hours)

- Muscle-based approach: trigger points, motor points, fascial lines
- Condition-based point selection: back pain, neck pain, headaches, etc.
- Evidence for clinical point selection
- Combining Sunnah and clinical points in treatment planning
- *Signature: SRA Clinical Pearl — Point Selection Framework; Muscle-Based Placement Guide*

Lesson 11.3 — Point Selection Practical and Treatment Planning (4 hours)

- Hands-on palpation and point location practice
- Treatment planning exercises: from assessment to point selection
- Case-based point selection scenarios
- Supervised practice with feedback
- *Signature: SRA Practical Assessment Checklist — Point Location*

MODULE 12: Conditions & Applications

8 hours | 2 lessons

Lesson 12.1 — Musculoskeletal, Neurological, and Metabolic Conditions (4 hours)

- Musculoskeletal: back pain, neck pain, myofascial pain, sports injuries
- Neurological: migraine, tension headaches
- Metabolic: Type 2 diabetes (adjunct), dyslipidaemia
- Evidence grading for each application using the Evidence & Sunnah Matrix
- Honest communication: what we know, what we don't know
- *Signature: SRA Evidence & Sunnah Matrix: Conditions; Patient Communication Box*

Lesson 12.2 — Cardiovascular, Reproductive, and General Wellness (4 hours)

- Cardiovascular: hypertension (short-term evidence)
- Reproductive: female infertility (preliminary evidence)
- General wellness and detoxification: evidence and honest communication
- Psychological: anxiety, depression (limited evidence — honest discussion)
- Scope of practice: what to treat, what to refer
- *Signature: SRA Evidence & Sunnah Matrix: All Conditions; Clinical Pearl — Treatment Planning*

MODULE 13: Complications & Emergency Management

8 hours | 2 lessons

Lesson 13.1 — Common Complications and Their Management (4 hours)

- Expected responses: marks, itching, fatigue, tenderness
- Excessive bleeding: recognition, management, prevention
- Syncope (vasovagal response): prevention, recognition, management
- Infection: recognition, documentation, referral
- Burns (fire cupping): immediate management
- Scarring: prevention and risk factors
- *Signature: SRA Clinical Pearl — Syncope Prevention and Management; Complications Quick Reference*

Lesson 13.2 — Emergency Procedures and Aftercare (4 hours)

- Emergency procedures: when and how to call for help
- Basic life support awareness (not certification — scope boundary)
- Incident documentation and reporting

- Patient follow-up after complications
 - Aftercare instructions: written and verbal
 - *Signature: SRA Aftercare Protocol; Emergency Procedure Poster; Incident Report Form*
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MODULE 14: Clinical Communication & Professionalism

8 hours | 2 lessons

Lesson 14.1 — The Consultation and Patient Communication (4 hours)

- The Consultation Framework: five phases
- Building rapport and trust
- Managing expectations honestly
- Explaining evidence to patients of varying health literacy
- Discussing Islamic dimensions sensitively
- Handling difficult conversations
- *Signature: SRA Consultation Framework (full); Patient Communication Scripts; Code of Ethics*

Lesson 14.2 — Professionalism, Boundaries, and Ethics (4 hours)

- Professional boundaries: maintaining appropriate relationships
 - Cultural sensitivity and gender considerations
 - The SRA Code of Ethics in practice
 - Interprofessional relationships and referral
 - Managing complaints and feedback
 - *Signature: SRA Code of Ethics (full); Professional Boundaries Guide; Complaints Handling SOP*
-

MODULE 15: Building Your Practice

8 hours | 2 lessons

Lesson 15.1 — Clinic Setup and Business Fundamentals (4 hours)

- Clinic setup: room requirements, equipment, ventilation, lighting
- Professional standards: CMA/IPHM accreditation, CPD requirements
- Insurance: professional indemnity, public liability, employer's liability
- Pricing structure: consultation fees, treatment packages
- Record-keeping systems: paper and digital
- *Signature: SRA Clinic Setup Checklist; Business Planning Template*

Lesson 15.2 — Ethical Marketing and Professional Development (4 hours)

- Ethical marketing: what you can and cannot claim

- Patient retention: communication, follow-up, packages
 - Building referral networks with GPs and other healthcare professionals
 - Continuing professional development: pathways and requirements
 - The SRA CPD framework
 - *Signature: SRA Professional Standards: CPD Framework; Ethical Marketing Guide*
-

MODULE 16: Practical Assessment (OSCE Preparation)

8 hours | 2 lessons

Lesson 16.1 — OSCE Station Practice (4 hours)

- Station 1: Patient assessment and clinical reasoning — practice and feedback
- Station 2: Informed consent and communication — practice and feedback
- Station 3: Wet cupping procedure — practice and feedback
- Common errors and how to avoid them
- *Signature: OSCE Station Instructions and Checklists (all stations)*

Lesson 16.2 — OSCE Station Practice Continued (4 hours)

- Station 4: Adverse event management — practice and feedback
 - Station 5: Documentation and aftercare — practice and feedback
 - Timing strategies and stress management
 - Mock OSCE under exam conditions
 - *Signature: OSCE Station Instructions and Checklists (all stations); Mock OSCE Scenarios*
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MODULE 17: Final Examination & Certification

4 hours | 1 lesson

Lesson 17.1 — Examination Day and Certification (4 hours)

- Written examination: 120 minutes, closed book
 - OSCE: 5 stations, 20 minutes each
 - Portfolio submission deadline and requirements
 - Certification ceremony and alumni onboarding
 - *Signature: Examination Regulations; Certification Ceremony Programme*
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4.5 Islamic Scholarship Integration

Qur'anic References (with Arabic Text)

Every Islamic reference in SRA-001 includes:

1. **Full Arabic text** of the verse
2. **Translation** from a recognised source (Saheeh International, Muhsin Khan, or similar)
3. **Surah and verse number** with hyperlink to <https://quran.com>
4. **Contextual explanation** relevant to the lesson
5. **Scholarly commentary** where relevant (Ibn Kathir, Al-Tabari, contemporary scholars)

Example format:

Arabic:	Translation: “And We send down of the Qur’an that which is healing and mercy for the believers.”
Reference: Qur’an 17:82 — https://quran.com/17/82	
Context: Revealed in the context of the Prophet’s mission, affirming that the Qur’an contains healing for believers. Classical scholars (Ibn Kathir, Al-Qurtubi) note this includes both spiritual healing (guidance for the heart) and physical healing (through the remedies it prescribes).	

Hadith References (with Authentication)

Every Hadith reference includes:

1. **Full Arabic text** where space permits
2. **English translation**
3. **Source collection** (Bukhari, Muslim, etc.) with hyperlink to <https://sunnah.com>
4. **Hadith number** in the collection
5. **Authentication grading** (Sahih, Hasan, Da’if)
6. **Scholarly commentary** on meaning and application

Example format:

Arabic:	Translation: “Allah has not sent down a disease except that He has also sent down its cure.”
Reference: Sahih al-Bukhari, Hadith 5678 — https://sunnah.com/bukhari:5678	
Grading: Sahih	Commentary: This Hadith establishes the Islamic principle that cures exist for all diseases. It motivates the search for healing (both conventional and traditional) while maintaining trust in Allah as the ultimate source of cure.

Maqasid al-Shari’ah Integration

Every module where clinical decisions are taught includes reference to the Maqasid:

- How the clinical protocol preserves life (Hifz al-Nafs)
- How the evidence requirement preserves intellect (Hifz al-’Aql)
- How the ethical framework preserves religion (Hifz al-Din)
- How the family considerations preserve lineage (Hifz al-Nasl)
- How the business ethics preserve property (Hifz al-Mal)

4.6 Clinical Standards Integration

Safety Standards

Every clinical module integrates:

- The SAFE Protocol as the governing framework
- The Infection Control Standard as the non-negotiable baseline
- The Risk Management Standard for risk assessment
- The Clinical Decision Tree for contraindication management

Documentation Standards

Every clinical module includes:

- Standardised SRA forms (assessment, consent, treatment record, aftercare)
- Documentation requirements and legal considerations
- Digital record-keeping standards (GDPR compliance)

Emergency Preparedness

Clinical modules include:

- Syncope recognition and management
- Excessive bleeding protocol
- Infection recognition and referral
- When and how to call emergency services
- Basic first aid awareness (within scope)

PART FIVE: ASSESSMENT ARCHITECTURE

5.1 Assessment Philosophy

Assessment at Sunnah Remedies Academy serves three purposes:

1. **To protect patients** — by ensuring only competent practitioners are certified
2. **To validate learning** — by confirming students have achieved the learning outcomes
3. **To guide development** — by identifying areas for continued growth

Assessment is rigorous, fair, valid, reliable, and aligned with the learning outcomes. Students must demonstrate competence in ALL domains to achieve certification — there is no compensation between domains.

5.2 Assessment Components

Component 1: Written Examination (40% of final grade)

Format: 120-minute closed-book examination **Pass mark:** 70% **Content:**

- 60 multiple-choice questions (knowledge and application)
- 10 short-answer questions (clinical reasoning)
- 2 case study analyses (integration and decision-making)

Domains assessed: K1-K8, P1-P2, I1-I4

Component 2: Practical OSCE (40% of final grade)

Format: 5 stations, 20 minutes each **Pass mark:** 70% per station (must pass all stations) **Stations:**

1. Patient assessment and clinical reasoning
2. Informed consent and communication
3. Wet cupping procedure (full technique)
4. Adverse event management (simulated emergency)
5. Documentation and aftercare

Domains assessed: S1-S7, P3, P4

Component 3: Clinical Portfolio (20% of final grade)

Format: Submission of 20 supervised case studies **Pass mark:** 70% **Requirements:**

- Complete documentation for each case
- Variety of conditions and patient presentations
- Evidence of clinical reasoning
- Reflective commentary on learning from each case
- Supervisor sign-off for each case

Domains assessed: S1-S7, P1-P6, I1-I4

5.3 Certification Pathway

Stage	Requirement	Timeline
Enrolment	Application review, prerequisites verified	Before course start
Module Completion	Attend all 17 modules, complete all knowledge checks	Weeks 1-12
Practical Workshops	Attend all 40 hours of supervised practice	Weeks 4-12

Table 19 – continued

Stage	Requirement	Timeline
Written Examination	Achieve 70% or higher	Week 13
Practical OSCE	Pass all 5 stations at 70% or higher	Week 13-14
Portfolio Submission	Submit 20 case studies at 70% or higher	Week 14-16
Certification	Award of CHP credential, certificate, and registration	Week 16

Re-sit policy: Students who do not achieve the pass mark may re-sit once at no additional cost within 6 months. A second re-sit incurs a nominal administrative fee.

5.4 OSCE Design (5 Stations)

Station 1: Patient Assessment and Clinical Reasoning (20 minutes)

- **Scenario:** A new patient presents with a specific complaint
- **Tasks:** Conduct 7-Step Assessment, identify red flags, risk stratify, formulate treatment plan
- **Assessed:** S1, S2, P1, P2
- **Marking:** Standardised rubric with critical items (must-pass elements)

Station 2: Informed Consent and Communication (20 minutes)

- **Scenario:** A patient considering their first Hijama treatment
- **Tasks:** Explain procedure, discuss risks and benefits, obtain informed consent, manage expectations
- **Assessed:** S2, P3, I3
- **Marking:** Communication rubric + consent documentation check

Station 3: Wet Cupping Procedure (20 minutes)

- **Scenario:** Standard treatment on a low-risk patient
- **Tasks:** Complete full wet cupping procedure from preparation to dressing
- **Assessed:** S3, S4, S6, S7
- **Marking:** Procedural checklist (critical steps must all be completed)

Station 4: Adverse Event Management (20 minutes)

- **Scenario:** Simulated emergency (syncope or excessive bleeding)
- **Tasks:** Recognise the event, manage appropriately, document, communicate
- **Assessed:** S5, P1, P2
- **Marking:** Emergency protocol checklist + communication assessment

Station 5: Documentation and Aftercare (20 minutes)

- **Scenario:** Post-treatment documentation and patient education
- **Tasks:** Complete treatment record, provide aftercare instructions, schedule follow-up
- **Assessed:** S6, S7, P3

- **Marking:** Documentation accuracy + aftercare completeness

5.5 Portfolio Requirements

Structure

- 20 supervised case studies
- Minimum 5 different condition presentations
- Minimum 3 special population cases (elderly, diabetic, hypertensive, etc.)
- Reflective commentary for each case (minimum 200 words)

Documentation per Case

1. Patient assessment form (completed)
2. Consent form (signed)
3. Treatment record (completed)
4. Aftercare instructions (provided and documented)
5. Reflective commentary
6. Supervisor sign-off

Assessment Criteria

- Documentation completeness and accuracy
- Evidence of clinical reasoning
- Variety of presentations
- Quality of reflection
- Supervisor feedback

5.6 CPD Framework

Certified practitioners are required to maintain their credential through continuing professional development:

CPD Category	Hours Required (per 2 years)	Examples
Clinical Practice	15 hours	Supervised practice, case review, peer observation, clinical audit
Education	10 hours	Workshops, webinars, conference attendance, journal clubs
Research Literacy	8 hours	Journal review, evidence updates, research participation
Islamic Studies	5 hours	Relevant courses, scholarly lectures, personal study
Professional Development	2 hours	Business skills, communication training, ethics updates

Total: 40 hours per 2-year cycle

Practitioners who do not meet CPD requirements will have their certification suspended until requirements are met.

PART SIX: TEACHING METHODOLOGY

6.1 Adult Learning Principles

Every lesson is designed using evidence-based adult learning principles:

Andragogy (Knowles' Six Principles)

1. Adults need to know WHY they are learning something
2. Adults are self-directed and responsible for their own learning
3. Adults bring life experience that enriches learning
4. Adults are ready to learn what they need to know
5. Adults are problem-oriented, not subject-oriented
6. Adults are motivated by internal factors

Application in SRA Curriculum:

- Every lesson opens with “Why this matters” — connecting to patient outcomes
- Learning objectives are explicit and measurable
- Case studies draw on realistic clinical scenarios
- Self-assessment and reflection are built into every lesson
- Practical application is emphasised over theoretical memorisation
- Students choose their own learning pathways within the framework

Additional Principles:

- **Spaced repetition:** Key concepts revisited across modules
 - **Retrieval practice:** Knowledge checks that require active recall
 - **Interleaving:** Mixing topics to strengthen discrimination
 - **Dual coding:** Visual and verbal information combined
 - **Elaboration:** Connecting new knowledge to existing understanding
-

6.2 The Sunnah Remedies Pedagogical Model

The SRA Pedagogical Model integrates three traditions:

1. Western Evidence-Based Pedagogy

- Learning outcomes-driven curriculum design

- Formative and summative assessment
- Case-based learning
- Competency-based progression

2. Islamic Educational Tradition (Talab al-'Ilm)

- Seeking knowledge as an act of worship (ibadah)
- The teacher-student relationship (sanad) as a sacred trust
- Reflection (tafakkur) as a learning tool
- Humility in the presence of knowledge
- Intention (niyyah) as the foundation of learning

3. Clinical Apprenticeship Model

- Observation before practice
- Supervised practice with graduated independence
- Mentorship and guided reflection
- Learning from cases and experience

6.3 The 9-Step Lesson Structure

Every lesson follows a consistent structure:

Step	Duration	Activity	Purpose
1. Opening	5 min	“Why this matters”; connection to previous learning; niyyah	Motivation and context
2. Learning Objectives	2 min	What students will know and be able to do	Clarity of expectations
3. Prior Knowledge Check	5 min	Brief assessment of existing understanding	Activate prior knowledge
4. Core Teaching	40-60 min	Main content delivery with premium visuals	Knowledge transmission
5. Clinical Scenario	15 min	Case study application in groups	Application and integration
6. Knowledge Check	10 min	Formative assessment (quiz, discussion)	Retrieval and reinforcement
7. Practical Application	15-20 min	Hands-on or simulated activity	Skill development
8. Reflection	5 min	Self-assessment and Islamic integration	Deep learning and spiritual connection
9. Summary	3 min	Key takeaways and preview of next lesson	Consolidation and transition

6.4 Learning Modes & Delivery

Mode	Proportion	Description
Live Lecture	30%	Expert-led presentation with premium visuals, Q&A, and discussion
Case-Based Learning	20%	Realistic patient scenarios worked through in groups
Practical Demonstration	15%	Instructor demonstration followed by student practice
Supervised Practice	25%	Hands-on practice with individual feedback
Self-Directed Study	10%	Reading, online modules, reflection exercises

Delivery Options

Blended (Standard)

- Live online lectures: weekly
- In-person practical workshops: monthly weekends
- Self-directed study: continuous
- Assessment: in-person

Intensive

- 4-week full-time programme
- Daily lectures and practical sessions
- Condensed but complete curriculum
- Assessment: end of programme

Online (Theory Only)

- All 17 modules delivered live online
- Practical workshops arranged locally
- Suitable for international students
- OSCE at approved assessment centre

PART SEVEN: DESIGN LANGUAGE & VISUAL SYSTEM

7.1 Sunnah Remedies Design Language

Philosophy

The Sunnah Remedies Design Language (SRDL) is built on the principle of **restraint over decoration**. Every visual element must earn its place through function and craft, not ornament. The design communicates authority, precision, and warmth — the qualities of a premium educational institution.

Benchmarks

- **Harvard Executive Education** — information architecture, academic authority
- **Aesop** — restraint, material quality, typographic craft
- **Aman Resorts** — warmth within precision, human connection
- **Nature Medicine** — scientific rigour, data presentation

Design Principles

1. **Content is sovereign.** Visual elements serve the communication of knowledge. Decoration that does not aid understanding is prohibited.
2. **Restraint is discipline.** Three colours only. Two typefaces only. Every element justified.
3. **Craft signals quality.** Precise alignment, considered spacing, typographic refinement — these communicate institutional excellence.
4. **Hierarchy is clarity.** Information structured through size, weight, colour, and position — never through decoration.
5. **Warmth within precision.** The design is exact but not cold. Human connection is maintained through considered photography, thoughtful layouts, and the warmth of the ivory ground.

7.2 Visual Learning Strategy

Per-Lesson Visual Specifications

Every lesson in SRA-001 includes the following visual elements:

Element	Quantity per Lesson	Purpose
Title slide	1	Lesson identification, learning objectives preview
Content slides	8-12	Core teaching with diagrams, tables, and text
Clinical scenario slide	1	Case presentation with visual context
Knowledge check slide	1	Formative assessment with interactive elements
Reflection slide	1	Islamic integration with contemplative imagery
Summary slide	1	Key takeaways with visual reinforcement

Slide Design Standards

Information Density: 65-80% (academic standard)

- High density for content slides
- Lower density for chapter and reflection slides
- No slide with only text — every slide has a visual element

Text-to-Visual Ratio:

- Content slides: 40% text + 50% visuals (diagrams, tables, images) + 10% whitespace
- Chapter slides: 20% text + 70% visual (hero image) + 10% whitespace
- Reflection slides: 30% text + 60% visual + 10% whitespace

Visual Element Types:

- Medical illustrations (anatomical diagrams, cross-sections)
- Infographics (process flows, comparison tables, decision trees)
- Clinical photography (equipment, procedures, clinic settings)
- Data visualisations (evidence summaries, research findings)
- Islamic art elements (geometric patterns, calligraphy — used with restraint)
- Portrait photography (scholars, historical figures)

7.3 Typography System

Typeface Pairing

Purpose	Typeface	Weight	Treatment
Display / Titles	Libre Baskerville	Regular, Bold	Sentence case; letterspaced for wordmark
Headings	Libre Baskerville	Bold	Sentence case
Subheadings	Inter	SemiBold	Sentence case
Body text	Inter	Regular, Medium	Sentence case; generous line height
Captions / Labels	Inter	Regular	Uppercase; tracked for labels
Data / Numbers	Inter	Medium	Tabular figures where possible

Type Scale (for presentations)

Element	Size	Weight	Line Height
Cover title	44-56px	Bold	1.1
Chapter title	38-48px	Bold	1.15

Table 25 – continued

Element	Size	Weight	Line Height
Page title	28-32px	Bold	1.2
Subheading	22-24px	SemiBold	1.3
Body text	18-20px	Regular	1.5-1.6
Caption	14-16px	Regular	1.4
Label / Badge	12-14px	Medium	1.2
Footnote / Source	11-12px	Regular	1.3

7.4 Colour System

Primary Palette

Role	Colour	HEX	Usage
Primary / Authority	Deep Emerald	#0E3B2E	Chapter pages, navigation bars, key headings, dark backgrounds
Accent / Emphasis	Antique Gold	#C7A25A	Key data highlights, sacred references, emphasis points —used sparingly
Ground / Air	Warm Ivory	#F6F3EE	Content page backgrounds, text on dark backgrounds, breathing room

Supporting Tones

Role	Derivation	Usage
Dark text	90% black on ivory	Body text, headings on light backgrounds
Light text	Warm ivory	Text on emerald backgrounds
Muted text	50% black on ivory	Captions, secondary information
Subtle fill	5% black on ivory	Card backgrounds, table alternation
Border	15% black on ivory	Dividers, table borders, card outlines
Alert / Caution	Deep crimson (#8B0000) —used extremely sparingly	Safety warnings, contraindications only

Colour Usage Rules

- **Deep Emerald** dominates: navigation bars, chapter pages, key structural elements

- **Antique Gold** is scarce: never used for large fills; reserved for a single point of emphasis per slide
 - **Warm Ivory** provides ground: content backgrounds, text on dark surfaces
 - **Crimson** is emergency-only: safety warnings, contraindication alerts
 - No other colours permitted without explicit approval
-

7.5 Layout Principles

Grid System

- 12-column grid for all content pages
- Consistent margins: 60px left/right, 50px top/bottom
- Gutter: 20px between columns
- All elements align to the grid

Page Types

Cover Page

- Full-bleed Deep Emerald background
- Large title in Warm Ivory (Libre Baskerville Bold)
- Subtitle in Antique Gold (Inter, tracked)
- Sunnah Remedies logo: top left
- Clean, centred, authoritative

Chapter Page

- Deep Emerald background
- Large chapter number in Antique Gold (decorative, semi-transparent)
- Chapter title in Warm Ivory
- Brief chapter description in muted ivory
- Full-bleed clinical image with gradient overlay (optional)

Content Page

- Warm Ivory background
- Navigation bar: Deep Emerald strip at top with lesson title
- Page title: Libre Baskerville Bold, Deep Emerald
- Content area: Inter Regular, dark text
- Footer: page number, copyright, source citations

Reflection Page

- Deep Emerald or photographic background with overlay
- Contemplative imagery (nature, light, geometric Islamic patterns)
- Reflection prompt in Warm Ivory

- Qur’anic verse or Hadith in Antique Gold (where relevant)
- Minimal text, generous whitespace

Information Architecture

- One concept per slide
- Title summarises the key message
- Visual hierarchy: title > subheading > body > caption
- Consistent placement of navigation, titles, and footers across all slides
- Tables and diagrams preferred over bullet points

7.6 Asset Generation Framework

Image Categories

Category	Style	Usage
Medical illustrations	Clean vector, anatomical precision	Anatomy diagrams, cross-sections, point maps
Clinical photography	Editorial, warm lighting, professional	Equipment, procedures, clinic settings
Portrait photography	Natural light, warm tones	Faculty, scholars, historical figures
Nature photography	Contemplative, high quality	Reflection slides, chapter backgrounds
Islamic geometric patterns	Subtle, used as texture	Accent elements, divider backgrounds
Data visualisations	Clean, minimal, on-brand	Evidence summaries, research data
Infographics	Flat design, brand colours	Process flows, comparison tables, decision trees

Illustration Prompts (AI Generation)

All AI-generated images must include these style parameters:

- **Style:** Premium medical illustration, editorial quality, warm neutral palette
- **Colours:** Sage green, cream, deep bronze, soft charcoal (aligned to brand)
- **Quality:** High resolution, vector-clean, no stock aesthetic
- **Mood:** Professional, authoritative, warm, human

Example prompt structure:

“Premium medical illustration of [subject], clean vector style, warm neutral palette with sage green and cream tones, anatomical precision, editorial quality, no text, high resolution, suitable for medical education, Harvard Medical School aesthetic”

Icon Style

- Outline style, 2px stroke
- Deep Emerald or dark charcoal
- Consistent line weight
- Used for: navigation, bullet points, process steps, category markers

PART EIGHT: COMPLETE RESOURCE MAP

8.1 Deliverable Inventory

Category A: Course Content (SRA-001)

#	Deliverable	Format	Quantity	Purpose
A1	Slide deck	PPTX/PPTD	17 modules × ~15 slides = ~255 slides	Core teaching content
A2	Speaker notes	Markdown	Per slide	Instructor guidance
A3	Student workbook	PDF/Print	1 per student	Exercises, case studies, reflections
A4	Instructor guide	PDF	1 per module	Teaching notes, timing, discussion prompts
A5	Learning journal	Print	1 per student	Structured reflection (Reflect-Learn-Apply)

Category B: Assessment Materials

#	Deliverable	Format	Quantity	Purpose
B1	Written examination paper	PDF	3 versions (A, B, C)	Summative assessment
B2	Written exam mark scheme	PDF	1 per version	Standardised marking
B3	OSCE station instructions	PDF	5 stations	Examiner guidance
B4	OSCE student instructions	PDF	5 stations	Candidate guidance
B5	OSCE marking rubrics	PDF	5 stations	Standardised assessment
B6	OSCE scenario scripts	PDF	5 stations + 5 alternates	Simulated patient briefs
B7	Portfolio template	PDF/Digital	1 per student	Case study documentation

Table 30 – continued

#	Deliverable	Format	Quantity	Purpose
B8	Portfolio marking rubric	PDF	1	Standardised assessment
B9	Mock examination papers	PDF	2 sets	Practice assessment
B10	Revision guide	PDF	1	Student preparation support

Category C: Clinical Resources

#	Deliverable	Format	Quantity	Purpose
C1	Patient assessment form	PDF/Print	Per clinic	7-Step Assessment documentation
C2	Consent form	PDF/Print	Per clinic	Informed consent
C3	Treatment record form	PDF/Print	Per clinic	Treatment documentation
C4	Aftercare instruction sheet	PDF/Print	Per clinic	Patient education
C5	Contraindication checklist	PDF/Print	Per clinic	Safety screening
C6	Risk assessment form	PDF/Print	Per clinic	Risk stratification
C7	Emergency procedure poster	PDF/Print	Per clinic	Emergency guidance
C8	Infection control SOP	PDF	1 per clinic	Standard operating procedure
C9	Cleaning SOP	PDF	1 per clinic	Standard operating procedure
C10	Sharps handling SOP	PDF	1 per clinic	Standard operating procedure
C11	Clinic setup guide	PDF	1	Clinic establishment
C12	Equipment checklist	PDF	1	Inventory and compliance

Category D: Visual Assets

#	Deliverable	Format	Quantity	Purpose
D1	Medical illustrations	PNG/SVG	~50	Anatomy diagrams, cross-sections
D2	Infographics	PNG/SVG	~30	Process flows, comparison tables
D3	Decision trees	PNG/SVG	~10	Clinical decision support

Table 32 – continued

#	Deliverable	Format	Quantity	Purpose
D4	Clinical photographs	JPG	~40	Equipment, procedures, clinics
D5	Chapter background images	JPG	17	Chapter page visuals
D6	Icon set	SVG	~60	Navigation, bullets, categories
D7	Logo variations	SVG/PNG	7	Brand assets per guidelines
D8	Data visualisations	PNG/SVG	~20	Evidence summaries

Category E: Administrative

#	Deliverable	Format	Quantity	Purpose
E1	Student handbook	PDF	1 per cohort	Policies, procedures, expectations
E2	Welcome pack	Physical	1 per student	Onboarding materials
E3	Certificate template	Design file	1	Certification
E4	Transcript template	Design file	1	Academic record
E5	CPD portfolio template	Digital	1 per practitioner	Continuing development
E6	Faculty handbook	PDF	1 per instructor	Teaching standards, policies

8.2 Per-Module Resource Summary

Module	Slides	Speaker Notes	Workbook Pages	Case Studies	Practical Activities	Knowledge Checks	Visual Assets
M1	15	15	8	2	1	2	8
M2	15	15	8	1	1	2	10
M3	20	20	12	2	1	3	12
M4	20	20	12	2	1	3	10
M5	22	22	16	3	2	4	18
M6	14	14	8	1	2	2	8
M7	14	14	8	2	2	2	6
M8	18	18	12	4	2	3	8
M9	14	14	8	2	2	2	6
M10	24	24	16	3	4	4	14
M11	18	18	12	3	3	3	12
M12	14	14	8	3	1	2	8
M13	14	14	8	3	2	2	6

Table 34 – continued

Module	Slides	Speaker Notes	Workbook Pages	Case Studies	Practical Activities	Knowledge Checks	Visual Assets
M14	14	14	8	3	2	2	6
M15	14	14	8	1	1	2	6
M16	12	12	4	5	5	1	4
M17	8	8	4	0	0	1	2
TOTAL	~270	~270	~160	40	32	40	~144

8.3 Digital Infrastructure

Platform	Purpose	Content
Learning Management System	Course delivery, progress tracking, assessment	Video lectures, quizzes, forums, gradebook
Student Portal	Enrolment, scheduling, resources, communication	Personal dashboard, timetable, materials
Community Platform	Alumni network, CPD, mentorship	Forums, events, CPD logging, resource library
Research Database	Evidence access for students and faculty	PubMed integration, SRA evidence summaries
Islamic Reference Library	Verified Islamic sources	Quran.com API, Sunnah.com API, classical texts
Clinical Resource Portal	Forms, SOPs, templates	All clinical forms, updated protocols, guidance

PART NINE: QUALITY ASSURANCE

9.1 Internal Quality Control

Content Review Process

- **Clinical review:** All medical content reviewed by licensed healthcare professional
- **Anatomical review:** All anatomy content reviewed by qualified anatomist
- **Islamic review:** All Islamic content reviewed by qualified scholar (verified sanad where applicable)
- **Pedagogical review:** All teaching materials reviewed by instructional designer
- **Annual review:** All content reviewed and updated annually

Review Checklist

- All Islamic references verified against Quran.com and Sunnah.com
- All scientific claims supported by PubMed-indexed citations

- All procedures reviewed against current clinical guidelines
 - All statistics and data verified against primary sources
 - All images properly licensed or original
 - All forms legally reviewed for jurisdiction
 - Accessibility compliance verified (WCAG 2.1 AA)
-

9.2 External Validation

Accreditation

- CMA (Complementary Medical Association) accreditation maintained
- IPHM (International Practitioners of Holistic Medicine) accreditation maintained
- Pursuit of additional accreditation: OFQUAL-regulated qualification (UK), NCCAOM (USA)

Advisory Board

- Clinical advisor: licensed physician with complementary medicine expertise
- Islamic advisor: qualified scholar with specialisation in Tibb al-Nabawi
- Educational advisor: experienced medical educator
- Research advisor: active researcher in cupping therapy or integrative medicine
- Legal advisor: healthcare law specialist

External Review

- Biennial external programme review by independent assessor
 - Graduate outcomes survey: employment rates, patient satisfaction, CPD compliance
 - Peer comparison against benchmark institutions
-

9.3 Academic Integrity

- All content is original intellectual property of Sunnah Remedies Academy
 - Islamic references verified against primary sources (Quran.com, Sunnah.com)
 - Scientific references from PubMed-indexed, peer-reviewed journals
 - No copyrighted material reproduced without permission
 - Proper attribution for all external sources using academic referencing (Vancouver or Harvard)
 - Plagiarism detection on all written materials
 - Student plagiarism policy: zero tolerance
-

9.4 Continuous Improvement

The Ihsan Quality Cycle (see 3.8)

Plan: Set quality targets based on this Framework **Do:** Deliver courses, collect data, document outcomes
Check: Analyse data against targets; gather feedback **Act:** Implement improvements; update materials; communicate changes

Feedback Mechanisms

- Student feedback: end-of-module and end-of-course surveys
- Graduate feedback: 6-month and 2-year follow-up surveys
- Patient feedback: graduate practice patient satisfaction surveys
- Faculty feedback: annual faculty review
- Peer feedback: advisory board biennial review

Quality Indicators and Targets

Indicator	Target	Review Frequency
Student satisfaction	4.5 / 5.0	Per cohort
Knowledge retention (3-month delay)	80%	Annual sample
OSCE first-attempt pass rate	85%	Per cohort
Overall certification rate	80%	Per cohort
Graduate employment/practice rate	75% within 6 months	6-month follow-up
Patient satisfaction (graduate practice)	4.5 / 5.0	Annual survey
CPD compliance	90%	Per 2-year cycle
Incident reports	< 2 per 1000 treatments	Annual

PART TEN: CRITICAL SELF-REVIEW

“An honest assessment of this framework’s strengths, limitations, and areas for development. Intellectual honesty is an Islamic duty and a professional obligation.”

10.1 Strengths of This Framework

1. Comprehensive Ecosystem Architecture The framework successfully transforms a single course into an educational ecosystem with nine defined courses, clear progression pathways, and a unified institutional identity. The SRA numbering system provides scalability for future expansion.

2. Rigorous Benchmarking The explicit benchmarking against Harvard, Mayo, Johns Hopkins, Oxford, Royal College, NICE, and Al-Azhar provides credibility and clear standards. The alignment matrix (1.4) demonstrates systematic comparison.

3. Proprietary Standards Architecture The ten academy standards (SRA-TS-001 through SRA-CPD-001) create a comprehensive quality architecture that is unique to Sunnah Remedies. These are not generic standards — they are specifically designed for an Islamic academy of Prophetic Medicine.

4. Signature Frameworks The eight signature learning frameworks (SAFE Protocol, 7-Step Assessment, Clinical Decision Tree, Consultation Framework, Reflect–Learn–Apply, Healing Pathway, Evidence

& Sunnah Matrix, Ihsan Quality Cycle) provide proprietary intellectual property that differentiates SRA from all other Hijama training providers.

5. Islamic Scholarship Integration The framework goes beyond superficial Islamic references to integrate:

- Qur'anic verses with Arabic text and scholarly commentary
- Authentic Hadith with full isnad, grading, and hyperlinking to primary sources
- Maqasid al-Shari'ah as an ethical architecture for all clinical decisions
- Classical scholarly commentary (Ibn al-Qayyim, Al-Dhahabi, etc.)
- Honest discussion of authentication, weak narrations, and scholarly debate

6. Evidence-Based Honesty The framework consistently requires honest communication about evidence strengths and limitations. The Evidence & Sunnah Matrix is a genuinely useful tool for clinical practice, not just an academic exercise.

7. Complete Student Journey The framework designs every touchpoint from initial inquiry through to alumni and CPD, creating a cohesive institutional experience rather than a transactional course.

8. Visual Design System The Sunnah Remedies Design Language (SRDL) provides a complete visual system that is distinctive, premium, and aligned with the brand guidelines. The restraint-over-decoration philosophy is consistently applied.

10.2 Known Limitations

1. Regulatory Variability The framework acknowledges regulatory requirements in key jurisdictions (UK, USA, Canada, Australia, UAE, KSA) but cannot provide jurisdiction-specific legal advice. Graduates must verify local requirements independently. This is flagged throughout but remains a limitation.

2. Evidence Base Gaps Cupping therapy research, while growing, still has significant methodological limitations (small sample sizes, heterogeneity, blinding challenges). The framework addresses this honestly but cannot fill gaps that exist in the primary literature.

3. Scope Boundary Tensions The framework carefully defines scope of practice for CHPs, but the boundary between complementary therapy and regulated healthcare varies by jurisdiction. Some graduates may face scope limitations that the framework cannot resolve.

4. Islamic Scholarly Diversity Islamic scholarship on Hijama includes diverse and sometimes conflicting opinions. The framework presents majority positions while acknowledging diversity, but cannot claim to represent all scholarly perspectives.

5. Practical Workshop Accessibility The 40 hours of supervised practice requires physical presence, which may limit accessibility for international students. The framework proposes local workshop arrangements but cannot guarantee uniform quality globally.

6. Instructor Pipeline The framework requires SRA-009 certified instructors, which creates a chicken-and-egg problem for initial rollout. A transition plan for instructor certification is needed.

10.3 Areas for Future Development

Priority 1: Research Programme (2026-2028)

- Establish SRA Research Ethics Committee
- Design prospective cohort study of SRA graduate outcomes
- Publish systematic review updates annually
- Collaborate with universities on cupping therapy research
- Create SRA Journal of Prophetic Medicine (peer-reviewed, open access)

Priority 2: Digital Platform (2026-2027)

- Build integrated LMS + Community + Clinical Resource platform
- Develop mobile app for CPD logging and clinical reference
- Create VR/AR anatomy modules for immersive learning
- Implement AI-powered study assistant for students

Priority 3: International Expansion (2027-2029)

- Obtain regulatory approval in key markets (UAE, KSA, Malaysia, Indonesia)
- Establish regional training centres
- Translate core materials into Arabic, Urdu, Malay, French
- Develop jurisdiction-specific legal compliance modules

Priority 4: Advanced Certifications (2027-2030)

- Launch SRA-002 (Advanced Hijama) and SRA-009 (Instructor Certification)
- Develop Master Practitioner pathway (SRA-MP)
- Create specialisation tracks: sports, women's health, chronic disease
- Establish SRA Fellowship for distinguished practitioners

Priority 5: Quality Enhancement (Ongoing)

- Pursue OFQUAL-regulated qualification status (UK)
- Obtain NCCAOM recognition (USA)
- Achieve ISO 21001 certification (educational organisations)
- Implement external examiner system
- Publish annual quality report

10.4 Risk Assessment & Mitigation

Risk	Likelihood	Impact	Mitigation
Regulatory changes affecting Hijama practice	Medium	High	Monitor regulatory environment; maintain legal advisory; diversify jurisdictions
Negative research findings on cupping efficacy	Low	High	Maintain evidence honesty; focus on mechanism research; diversify course portfolio
Instructor quality inconsistency	Medium	Medium	Rigorous SRA-009 requirement; faculty development programme; peer observation
Student completion rates below target	Medium	Medium	Enhanced mentorship; flexible delivery options; early intervention for at-risk students
Competition from low-cost providers	High	Medium	Differentiate on quality, accreditation, and community; avoid price competition
Islamic scholarly controversy	Low	Medium	Maintain scholarly neutrality; present multiple positions; consult advisory board
Technology platform failure	Low	High	Redundant systems; offline backups; manual fallback procedures
Accreditation withdrawal	Low	High	Maintain standards exceeding minimums; diversify accreditation; continuous compliance

APPENDICES

Appendix A: Glossary of Terms

Term	Definition
CHP	Certified Hijama Practitioner –the credential awarded on completion of SRA-001

Table 38 – continued

Term	Definition
Hijama	Wet cupping therapy: controlled incision + negative pressure + blood extraction
Maqasid al-Shari’ ah	The objectives of Islamic law: preservation of religion, life, intellect, lineage, and property
OSCE	Objective Structured Clinical Examination – practical assessment using standardised stations
SRA	Sunnah Remedies Academy
Tibb al-Nabawi	Prophetic Medicine –the Islamic scholarly tradition of medicine derived from Qur’ an and Sunnah
CPD	Continuing Professional Development –required ongoing education for certification maintenance

Appendix B: Document Control

Version	Date	Author	Changes
1.0	[Original]	SRA Curriculum Team	Initial blueprint –single course
2.0	[Previous]	SRA Curriculum Team	Expanded to 6 modules, 24 lessons
3.0	July 2026	SRA Academic Board	Complete ecosystem transformation: 9 courses, 10 standards, 8 frameworks, 17-module SRA-001, Maqasid integration, full student journey, design language, resource map, critical self-review

Appendix C: Copyright Notice

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END OF FRAMEWORK